

EthicHawks

FORENSIC GOVERNANCE & COMPLIANCE CONSULTANCY

CONSOLIDATED FORENSIC OVERSIGHT REPORT

Systemic Regulatory Capture • Institutional Failure • Corporate Misrepresentation
Judicial Obstruction • Bad Faith Litigation • The Chilling Effect on Every Future Whistleblower

Presented to:	The Honourable Speaker of the National Assembly
For referral to:	Portfolio Committee on Health; Portfolio Committee on Employment and Labour; Portfolio Committee on Trade, Industry and Competition; Portfolio Committee on Justice and Correctional Services; Standing Committee on Public Accounts (SCOPA); Portfolio Committee on Police
Subject:	Systemic regulatory capture affecting Sr. Wilbur William Matthee, a protected whistleblower who exposed a Priority 1 clinical risk at Medscheme Holdings / GEMS, and the chilling effect on all future whistleblowers in South Africa, with urgent implications for the National Health Insurance (NHI) governance architecture.
Prepared by:	EthicHawks Forensic Governance & Compliance Consultancy – on behalf of Sr. Wilbur William Matthee
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Classification:	For Parliamentary Oversight Use — Protected Disclosure under PDA Section 8

“The NHI will not fail because of a lack of money. It will fail because the institutional architecture supposed to protect it from capture, corruption, and collapse is already broken – and we have done nothing to fix it.”

CONFLICT OF INTEREST DECLARATION AND ETHICAL STATEMENT

Honourable Speaker and Portfolio Committees, this report is submitted in the public interest by a whistleblower who has suffered severe occupational detriment, destitution, and homelessness as a direct and documented consequence of the conduct set out herein. The author and complainant, Sr. Wilbur William Matthee, declares the following interests and ethical commitments in the spirit of full transparency required by the Constitution of the Republic of South Africa, 1996, the Protected Disclosures Act 26 of 2000, and the ethical codes of the compliance and nursing professions.

1. Identity of the Complainant and Author

The complainant and the author of this report are the same person: Wilbur William Matthee, a Specialist Registered Nurse (SANC Registration No. 15831886), holder of a Postgraduate Diploma in Health Services Management (NQF Level 8, awarded with distinction, Cape Peninsula University of Technology), and the Founder of EthicHawks Forensic Governance and Compliance Consultancy. He is the subject of the retaliation, the unlawful salary deduction, the constructive dismissal, and the regulatory and judicial failures documented in this report. He appears before Parliament self-represented, without legal representation, because he is destitute. This is not a third-party report. It is a primary source testimony from the person who lived every failure documented herein.

2. Nature of the Interest – Personal and Professional

The complainant has a direct personal and financial interest in the remedies sought in this report. Those interests include, but are not limited to:

- Repayment of the unlawfully deducted salary of R14,811.77 (plus interest and penalties);
- Compensation for occupational detriment under the Protected Disclosures Act 26 of 2000;
- Compensation for automatically unfair constructive dismissal under the Labour Relations Act 66 of 1995;
- Correction of his statutory employment records (UI-19, Certificate of Service) to reflect constructive dismissal rather than voluntary resignation;
- Remediation of the systemic Gexus-DMS clinical risk affecting 1.9 million GEMS beneficiaries.

This interest is fully declared and is not concealed. The complainant makes no apology for seeking remedy for the harm he has suffered. He separates his personal claim for damages – which is properly before the Labour Court – from his constitutional petition for parliamentary oversight, which is the subject of this report. The personal interest does not diminish the public interest in the systemic failures documented herein. A whistleblower who has been destroyed by a system is not disqualified from asking Parliament to fix that system.

3. The Complainant’s Professional and Ethical Framework

Framework	Status	Relevance
South African Nursing Council (SANC)	Registered Nurse Specialist – currently deregistered due to inability to pay annual fees caused by the employer’s unlawful deduction.	The deregistration itself is evidence of the harm suffered and confirms the direct causal chain from the unlawful deduction.

Framework	Status	Relevance
Compliance Institute of Southern Africa (CISA)	Pledged to the Code of Ethical and Professional Conduct; designation CPrAc(SA) – pending final certification modules.	Formally committed to the ethical standards of the compliance profession, including independence, objectivity, and confidentiality.
Protected Disclosures Act 26 of 2000	The complainant is a protected whistleblower who made disclosures in good faith about a systemic clinical risk.	This report is itself a protected disclosure under Section 8 of the PDA – a disclosure to Parliament as an appropriate authority.
Constitution of the Republic of South Africa, 1996	The complainant invokes Sections 10 (dignity), 26 (housing), 27 (food, water, healthcare), 33 (just administrative action), 34 (access to courts), and 56 (right to petition Parliament).	This report is a constitutional petition under Section 56. The remedies sought are constitutionally grounded.

4. EthicHawks – The Consultancy

EthicHawks Forensic Governance and Compliance Consultancy is the professional vehicle through which the complainant conducts forensic governance analysis. EthicHawks is not acting as a commercial entity in this report. No third party has commissioned, funded, or influenced this report. EthicHawks is receiving no payment for this report. The complainant was an employee of Medscheme Holdings, which terminated his employment constructively on 12 December 2025. There is no ongoing commercial relationship with any party mentioned in this report.

The complainant's prospective commercial interest – if any – is limited to the forensic governance architecture he has proposed for the NHI. That proposal is a matter of public policy submitted in the public interest, not a tender or commercial solicitation. Any implementation of those proposals would be subject to a competitive process in which the complainant would stand in the same position as any other qualified compliance professional. This is not a conflict. It is a professional opinion submitted in the public interest.

5. The Employer's Anticipated Response – And Why It Fails

The complainant anticipates that the employer and its legal representatives – Deneys Reitz Inc. – may attempt to use this Conflict of Interest Declaration to discredit the report by characterising it as “self-serving.” That argument is answered in advance on four grounds:

- No one else has done this work. The complainant has compiled a 526-document evidentiary bundle because no regulator, no court, and no law firm has done so. The absence of any other party with the resources, access, or incentive to compile this record is itself evidence of systemic failure.
- The documentary record speaks for itself. The Root Cause Analysis (RCA) was generated by Medscheme's own systems. The payslip was generated by Medscheme's own payroll. The CRO suppression email (18 December 2025) was written by the employer's own Chief Risk Officer. The false UI-19 was submitted by the employer's own HR official. These documents are not the complainant's opinion. They are the employer's own records.
- The complainant's destitution is not a conflict – it is the consequence of the conduct documented. A destitute whistleblower is not less credible. He is more credible, because he has nothing to gain from fabrication and everything to lose from continued destitution.
- The complainant has invited independent verification of every fact. The full evidentiary bundle is available to any Portfolio Committee, any parliamentary legal officer, and any independent

forensic investigator that Parliament may appoint. This is not the behaviour of someone seeking to conceal.

6. The Public Interest Overrides Any Private Interest

The complainant's personal claim for damages (R14,811.77 plus interest) is less than 0.00007% of the NHI's projected annual budget of R200 billion. It is not the subject of this report. This report is about the systemic collapse of accountability that enabled that unlawful deduction to happen without consequence – and that will enable far greater losses inside the NHI Fund unless Parliament acts.

The public interest in this report is overwhelming: a systemic clinical risk affecting 1.9 million GEMS members remains unaddressed; fifteen statutory, regulatory, and judicial bodies have failed to provide a remedy; and the NHI is being built on governance architecture that this case proves is already broken. Section 56 of the Constitution guarantees the right of any citizen to petition Parliament. That right is not conditional on the petitioner having no personal interest in the subject matter. On the contrary, citizens with direct experience of institutional failure are often the most credible and necessary petitioners.

7. Declaration Under Oath

I, Wilbur William Matthee, declare under oath that: (1) The facts set out in this report are true and correct to the best of my knowledge and belief, and are supported by primary source documents referenced throughout. (2) I have a direct personal and financial interest in the remedies sought, which is fully declared. (3) The public interest in the systemic failures documented herein is distinct from and outweighs any private interest. (4) No third party has commissioned, funded, or influenced this report. (5) I have not been induced to make this report by any person or entity. I make this report of my own free will, in exercise of my constitutional right under Section 56 of the Constitution. (6) I am aware that making false statements in a submission to Parliament may be a criminal offence. I have not made any false statements. (7) The audio recordings and all other primary source documents referenced in this report are available to Parliament upon request.

Signed at Cape Town, June 2026.

Wilbur William Matthee | RN | BTech General Nursing | PGDip Health Services Management (NQF 8) | Founder, EthicHawks Forensic Governance & Compliance Consultancy

Email: info@ethichawks.co.za | This Conflict of Interest Declaration is an integral part of the Consolidated Forensic Oversight Report and must be read together with the foreword, executive summaries, and evidentiary sections that follow.

TABLE OF CONTENTS

SECTION	TITLE
Conflict of Interest Declaration	Conflict of Interest Declaration and Ethical Statement
Table of Contents	Table of Contents
List of Acronyms	List of Acronyms and Abbreviations
Glossary	Glossary of Key Legal Terms
Foreword	The NHI Will Be Built on the Bones of This Whistleblower
Part One	Harrowing Introduction – The Chilling Effect on the Floor
Part Two	Methodology and Scope of This Report
Part Three	Master Chronology – Key Events at a Glance
Part Four	Summary of Relief Sought from Parliament
Part Five	Scorecard of Failed Institutions (15 Bodies)
Part Six	Note to the Portfolio Committees – Specific Referrals
Part Seven	Executive Summary for Each Portfolio Committee
Part Eight	The Whistleblower and the Systemic Clinical Risk
Part Nine	The Four Health Regulators – A Cartel of Silence
9.1	Council for Medical Schemes (CMS)
9.2	Health Professions Council of South Africa (HPCSA)
9.3	South African Nursing Council (SANC)
9.4	South African Pharmacy Council (SAPC)
Part Ten	The Principal Scheme – GEMS: Complicity through Silence
Part Eleven	Department of Employment and Labour – Administrative Abandonment
Part Twelve	The B-BBEE Commission – Declination of Fronting Complaint
Part Thirteen	The South African Human Rights Commission – Chapter 9 Abdication
Part Fourteen	The South African Police Service – Refusal to Investigate
Part Fifteen	The Commission for Conciliation, Mediation and Arbitration (CCMA)
Part Sixteen	The Whistleblower House – Flawed Assessment and Abandonment
Part Seventeen	The Judiciary – Judicial Obstruction and Abuse of Process
17.1	High Court – The Gag Order and Fraud on the Court
17.2	Labour Court – Portal Obstruction and Default Judgment Delay
17.3	Registrar Misconduct and POPIA Breach
Part Eighteen	Corporate Misrepresentation – AfroCentric's False Annual Report 2025
Part Nineteen	The JSE and FSCA – Regulatory Complicity and Chilling Effect
Part Twenty	Bad Faith Litigation and Attrition – The Corporate Playbook

SECTION	TITLE
Part Twenty-One	The NHI Governance Gap – Linking Findings to the National Health Insurance
Part Twenty-Two	Status of Parallel Proceedings (Litigation Update)
Part Twenty-Three	Conclusion – Why Parliament Must Act Now
Part Twenty-Four	Recommendations to Parliament
Part Twenty-Five	Final Prayer for Relief
Epilogue	The Human Face Behind This Report
Index of Annexures	Index of Annexures / Consolidated Evidence Bundle
Signature Block	Signature Block and Commissioner of Oaths

LIST OF ACRONYMS AND ABBREVIATIONS

ACRONYM	FULL DESCRIPTION
BCEA	Basic Conditions of Employment Act 75 of 1997
B-BBEE	Broad-Based Black Economic Empowerment Act 53 of 2003
CCMA	Commission for Conciliation, Mediation and Arbitration
CIPC	Companies and Intellectual Property Commission
CISA	Compliance Institute of Southern Africa
CMS	Council for Medical Schemes (South Africa – note: distinct from the US agency of the same name)
CPUT	Cape Peninsula University of Technology
DEL	Department of Employment and Labour
EEA	Employment Equity Act 55 of 1998
FSCA	Financial Sector Conduct Authority
GEMS	Government Employees Medical Scheme
HPCSA	Health Professions Council of South Africa
IPID	Independent Police Investigative Directorate
IRBA	Independent Regulatory Board for Auditors
JSE	Johannesburg Stock Exchange
LRA	Labour Relations Act 66 of 1995
MSA	Medical Schemes Act 131 of 1998
NDOH	National Department of Health
NHI	National Health Insurance Act 20 of 2023
NQF	National Qualifications Framework
NPA	National Prosecuting Authority
OCJ	Office of the Chief Justice
OHSC	Office of Health Standards Compliance
PAIA	Promotion of Access to Information Act 2 of 2000
PAJA	Promotion of Administrative Justice Act 3 of 2000
PDA	Protected Disclosures Act 26 of 2000
POPIA	Protection of Personal Information Act 4 of 2013
SANC	South African Nursing Council
SAPC	South African Pharmacy Council
SAPS	South African Police Service
SCOPA	Standing Committee on Public Accounts
SENS	Stock Exchange News Service
SIU	Special Investigating Unit
TWBH	The Whistleblower House

GLOSSARY OF KEY LEGAL TERMS

TERM	DEFINITION
Automatically unfair dismissal	A dismissal that is deemed unfair by law regardless of the employer's reasons, including dismissal for making a protected disclosure (LRA Section 187(1)(h)). Compensation may be awarded up to 24 months' remuneration.
Biowatch principle	Constitutional principle established in Biowatch Trust v Registrar, Genetic Resources 2009 (6) SA 232 (CC) that state organs should not be awarded costs against private litigants who raise constitutional issues in good faith. The principle is intended to prevent costs from having a chilling effect on constitutional litigation.
Constructive dismissal	Occurs when an employer makes continued employment so intolerable that a reasonable employee has no choice but to resign (LRA Section 186(1)(e)). The resignation letter must clearly invoke this basis; a generic resignation may be treated as voluntary.
Ex parte	A court application made by one party in the absence of the other. Courts grant ex parte relief only where there is genuine urgency or a risk that notice will defeat the relief. Full and frank disclosure of all material facts – including any opposition – is an absolute requirement. Concealment of an answering affidavit from an ex parte court constitutes a fraud on the court.
Forensic governance	The deliberate architecture of systems that make misconduct detectable before it causes harm, accountability structures that function under pressure and without external prompting, and escalation pathways from anomaly detection to institutional oversight. Forensic governance is distinguished from compliance: compliance asks whether rules are followed; forensic governance asks whether misconduct is structurally impossible to hide.
Fronting (B-BBEE)	A practice prohibited under the B-BBEE Act in which a Black person or entity is used to satisfy B-BBEE requirements without that person or entity receiving the economic benefits, management control, or decision-making authority reasonably associated with the position or stake (B-BBEE Act Section 1(c)). Fronting is a criminal offence under Section 26B.
Induced destitution	The deliberate use of financial attrition – unlawful salary deductions, withholding of exit documents, suppression of UIF contributions, delay of legal processes – to exhaust a whistleblower's resources and coerce capitulation. Not a statutory term but a pattern identified by this report as a systemic weapon against self-represented litigants.
Mandamus	A court order compelling a public official or regulatory body to perform a statutory duty that it has failed to perform. Available under PAJA Section 8(1)(c) where a decision-maker has unreasonably delayed a decision or unlawfully refused to make one.
Occupational detriment	Any act or omission that prejudices an employee in respect of any aspect of employment, including dismissal, suspension, demotion, harassment, intimidation, transfer, or being subjected to civil or criminal proceedings (PDA Section 1). Includes indirect detriment such as the blocking of professional registration.
Perjury	The offence of making a false statement under oath in a legal proceeding. In South Africa, perjury is a common law offence carrying a maximum sentence of imprisonment without the option of a fine. A false statement does not need to be the direct cause of a court decision to constitute perjury.

TERM	DEFINITION
Protected disclosure	A disclosure made in good faith by an employee to an appropriate authority about the conduct of an employer that constitutes a criminal offence, failure to comply with a legal obligation, miscarriage of justice, endangerment of health or safety, or unfair discrimination (PDA Section 1). Once a disclosure is protected, any occupational detriment imposed as a consequence is automatically unfair under LRA Section 187(1)(h).
Rule nisi	A conditional court order requiring the respondent to appear and “show cause” why a final order should not be granted on a specified return date. Until the return date, the rule nisi operates as an interim order with the full force of a court order. Extension of a rule nisi to a date nearly one year from grant, without a substantive hearing, is exceptional and requires strong justification.
Rule 6(8) anticipation	A procedure under the Uniform Rules of Court allowing a party to bring a rule nisi back to court before its return date on 24 hours’ notice, typically where circumstances have changed or new facts have emerged.
Scale C costs	The highest scale of legal costs in South African litigation, typically reserved for complex commercial matters or cases of special difficulty. Awarding Scale C costs against a self-represented litigant – particularly one whose constitutional rights are in issue – may be unconstitutional in light of the Biowatch principle.
Section 162 delinquency application	A court application under the Companies Act 71 of 2008 seeking a declaration that a director is delinquent. A delinquent director is disqualified from serving as a director for a period determined by the court (minimum seven years). The application may be brought by any affected person and is an important corporate accountability tool.
Wilbur Method™	A forensic governance framework developed by the complainant during and after his employment at Medscheme, identifying the pattern by which systemic integration gaps between clinical platforms generate cascading false data, contaminate actuarial models, and expose beneficiaries to clinical risk – while remaining invisible to standard compliance reporting.

FOREWORD

The NHI Will Be Built on the Bones of This Whistleblower – Unless Parliament Intervenes Now

I. The Promise That Cannot Survive the System We Have

The National Health Insurance Act 20 of 2023 is anchored in an unambiguous constitutional promise. Section 27 of the Constitution of the Republic of South Africa, 1996 guarantees every person the right of access to health care services. The NHI is designed to be the instrument of that promise – a single purchaser and single payer, pooling risk across more than 60 million people, managing a budget conservatively projected to exceed R200 billion annually.

But here is the truth that no government report, no actuarial model, and no parliamentary briefing has yet confronted with the forensic granularity this case demands:

The NHI will not fail because of a lack of money, or a lack of political will, or a lack of clinical capacity. The NHI will fail because the institutional architecture that is supposed to protect it from capture, corruption, and collapse is already broken – and we have done nothing to fix it.

This forensic report is not, at its core, about the NHI. It is about a single whistleblower: a Specialist Registered Nurse employed by Medscheme Holdings, the accredited administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants. He identified a systemic clinical risk inside a data platform. He refused to use demonstrably false data to interrogate an elderly patient. He made a protected disclosure in terms of the Protected Disclosures Act 26 of 2000. He was retaliated against – methodically, systematically, and with institutional complicity – until he became destitute and homeless.

He then approached, in sequence, every regulator, oversight body, and court that will be tasked with overseeing the NHI Fund once it becomes operational. The Council for Medical Schemes (CMS). The Health Professions Council of South Africa (HPCSA). The South African Nursing Council (SANC). The South African Pharmacy Council (SAPC). The Department of Employment and Labour (DEL). The B-BBEE Commission. The South African Human Rights Commission (SAHRC). The South African Police Service (SAPS). The Commission for Conciliation, Mediation and Arbitration (CCMA). The Johannesburg Stock Exchange (JSE). The Financial Sector Conduct Authority (FSCA). The Western Cape High Court. The Labour Court.

Almost every single one of them declined jurisdiction, remained silent, or actively obstructed him.

If this is how the existing system treats a whistleblower who exposed a clinical risk in an administrator servicing a government medical scheme, what will happen when the NHI Fund – with its R200 billion annual budget, its single-purchaser monopoly, its accreditation bottleneck, its one-sentence fraud architecture, and its undefined investigation unit – becomes operational? The answer is not speculation. It is forensic prediction. The same institutions that failed this whistleblower will fail the NHI. The same regulatory capture that protected Medscheme will protect NHI contractors. The same judicial obstruction that silenced this nurse will silence the next whistleblower who tries to report corruption inside the NHI Fund.

II. The Three Structural Gaps – Already Visible, Already Destructive

The National Health Insurance Act contains three structural vulnerabilities that governance experts have warned about since its enactment. This report does not merely identify them in the abstract. It proves

they are already destroying lives in the existing system – and therefore will be catastrophic under the NHI.

Gap One: The Accreditation Bottleneck (NHI Act Section 39)

What the Act says: No provider may be reimbursed by the Fund unless accredited. Accreditation requires certification by the Office of Health Standards Compliance (OHSC) – an institution already documented by the Auditor-General as capacity-constrained, under-resourced, and incapable of processing the accreditation volume the NHI will require.

What the whistleblower's case proves: The B-BBEE fronting complaint (Case 5/02/2026) reveals a corporate strategy of title suppression: using a Black NQF Level 8 professional's credentials for scorecard points while downgrading his official title on statutory documents to avoid paying for his expertise. The same dynamic will infect NHI accreditation. When financial pressure meets an accreditation bottleneck, the predictable outcome is ghost accreditation (certification without genuine compliance assessment) and fronting (accredited entities serving as conduits for unaccredited providers). The B-BBEE Commission, the institution that would theoretically investigate this under the NHI, declined to investigate in the whistleblower's case, refused to use its Section 13K subpoena powers, and treated fronting as a labour dispute – an error of law that will be repeated when NHI accreditation fronting is reported.

Gap Two: The One-Sentence Fraud Architecture (NHI Act Section 40)

What the Act says: "The information architecture must include a fraud and risk management mechanism." Eleven words. For a system that will process millions of claims and determine health benefits for 60 million people – and that will become the single most concentrated target for health care fraud in South African history.

What the whistleblower's case proves: The Gexus-DMS integration gap – a structural system error that generated false non-adherence letters to chronic GEMS patients, corrupted clinical data, and inflated actuarial risk loading – was reported to the Council for Medical Schemes. The CMS declined jurisdiction twice. The Deputy Information Officer of Medscheme swore under oath that no IT incident had been reported, directly contradicting the internal Root Cause Analysis. The employer's Integrated Annual Report 2025 claimed "no material ESG incidents" while this gap remained unaddressed. The JSE and FSCA allowed that report to stand. Without a mandatory AI Audit Protocol and a fraud detection architecture with independent oversight, the NHI's information platform will be vulnerable to exactly this kind of structural manipulation – except on a scale of R200 billion.

Gap Three: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

What the Act says: The CEO of the NHI Fund must establish a Risk Management and Fraud Prevention Investigation Unit. The Act specifies no professional competency standards, no independence safeguards, no escalation protocols, no mandatory reporting lines, and no consequences for non-compliance.

What the whistleblower's case proves: On 9 December 2025, the complainant escalated a Priority 1 Material Risk Report – with 38 attachments, a fully documented Root Cause Analysis, and a formal section 60 Employment Equity Act personal liability warning addressed to the Board of AfroCentric – to the Board of Directors. The Board did not respond. The Chief Risk Officer (Monwabisi Kula), whose unit should have escalated the report immediately, instead instructed the complainant in writing: "Can I please ask you to stop sending these e-mails to our directors? Sending further e-mails only gives them an indication of impatience and unreasonableness on your part." This is active suppression of a material risk report by the very official whose unit should have escalated it.

The Zondo Commission documented this exact failure pattern across state-owned entities. A unit with a broad mandate, no professional standards, and no independence safeguards is not an oversight instrument. It is an oversight symbol. And an oversight symbol will not prevent the loss of R200 billion.

III. A Direct Appeal to the Speaker and the Portfolio Committees

Honourable Speaker, the letter from Deneys Reitz Inc. to the Speaker's Office – threatening the whistleblower with contempt of court for petitioning Parliament – reveals the corporate playbook. The interim interdict obtained by concealment of the answering affidavit reveals the judicial capture. The silence of every regulator reveals the regulatory collapse.

This is not a labour dispute. It is a governance crisis. And it is a definitive proof that South Africa is not ready for the National Health Insurance. The Portfolio Committees must act now. The evidence is before you. The time to act is now.

Prepared by EthicHawks Forensic Governance & Compliance Consultancy | June 2026

PART ONE: HARROWING INTRODUCTION – THE CHILLING EFFECT ON THE FLOOR

Honourable Speaker, this report is not a legal argument. It is not a petition for a court order. It is not an application for relief. It is a forensic record of what institutional failure looks like on the floor – inside the life of a single human being who did everything the law asked of him and was destroyed for it, methodically, completely, and with the active or passive participation of fifteen institutions that were each created precisely to prevent this outcome.

The whistleblower, a Specialist Registered Nurse (NQF Level 8) employed by Medscheme Holdings – the administrator for the Government Employees Medical Scheme (GEMS), which serves 1.9 million public servants – identified a systemic integration gap between the Gexus and DMS data platforms. This gap generated false non-adherence letters to chronic patients. The Root Cause Analysis, generated by Medscheme's own internal systems, confirmed the structural design flaw.

Confronted with a demand to use demonstrably false data to interrogate a 75-year-old GEMS beneficiary about medication adherence she was in fact maintaining, the complainant refused. His refusal was not an act of insubordination. It was an act of professional and ethical integrity. It was, in the language of the law, a protected disclosure under Section 1 of the Protected Disclosures Act 26 of 2000 (PDA): a disclosure made in good faith about conduct that endangers the health or safety of a patient. Section 1 of the PDA provides that any occupational detriment imposed as a result of a protected disclosure constitutes an automatically unfair dismissal under Section 187(1)(h) of the Labour Relations Act 66 of 1995 (LRA).

That refusal – and the formal protected disclosure that followed on 23 October 2025 – triggered a cascade of retaliation that this report documents across fifteen statutory, regulatory, and judicial bodies, not one of which provided a remedy. This section introduces that cascade in human terms, before the forensic detail of each institution is examined.

The Retaliation: A Chronological Record of Institutional Violence

The retaliation was not subtle, not gradual, and not deniable. Each act is documented by primary source evidence – the employer's own records, its own emails, its own payslips, its own sworn affidavits, and in one case an audio recording made by the complainant himself.

- **Unlawful salary deduction:** On 23 July 2025, Medscheme deducted R14,811.77 from the complainant's salary in a single pay cycle. This represented 40.47% of his gross monthly salary – exceeding the 25% cap imposed by Section 34(2) of the Basic Conditions of Employment Act 75 of 1997 (BCEA) by 15.47 percentage points. The deduction was made without the written consent required by BCEA Section 34(1), without a court order, and without the consultation required by the employer's own deduction policy.
- **Gender-identity harassment:** On 23 September 2025, the complainant was referred to by the name "Wilma" in a Microsoft Teams message by a colleague. This was not a mistake. It was a deliberately gendered insult targeted at a male professional. It is documented in writing. It constitutes harassment on the basis of gender under Section 6 of the Employment Equity Act 55 of 1998 (EEA).
- **Retaliatory formal charge:** On the same day the complainant filed a formal discrimination complaint (25 September 2025), the employer loaded double unpaid leave against his record. The temporal coincidence is not coincidental. It is documented. A formal insubordination charge was subsequently filed against him.
- **Recorded admission of retaliatory intent:** On 31 October 2025, a manager stated in a recorded meeting: "You made a case of harassment against me, so I can make a case of harassment

against you.” This is not an allegation. It is a direct admission of retaliatory intent, preserved on audio and transcribed.

- **Suppression of the Priority 1 Material Risk Report:** On 18 December 2025, the Chief Risk Officer (Monwabisi Kula) instructed the complainant in writing to “stop sending these e-mails to our directors,” characterising further escalation as evidence of “impatience and unreasonableness.” This constitutes active suppression of a material risk report by the very official whose unit should have escalated it.
- **Constructive dismissal:** On 12 December 2025, the complainant resigned. His resignation letter explicitly cited Section 186(1)(e) of the LRA – constructive dismissal. The employer subsequently submitted a UI-19 form to the Department of Employment and Labour classifying his departure as Code 6 (Resigned) rather than Code 7 (Constructive Dismissal). This is a false statutory declaration. It deprived the complainant of UIF benefits at the moment of his greatest vulnerability.
- **Sworn attack on his dignity and disability:** On 10 December 2025 – the day after the Priority 1 Material Risk Report was escalated to the Board – the employer’s CCMA opposing affidavit, sworn by Xolile Manyati, described the complainant’s clinically diagnosed severe depression and PTSD as a “cynical excuse.” This sworn statement is itself a subject of a potential perjury referral.

The Human Consequences: What Institutional Failure Looks Like

The consequences of this retaliation, amplified by the silence of every regulator and the obstruction of every court process, are not abstract. They are the lived reality of a human being.

- The complainant has been homeless since 2 April 2026.
- He has had no running water since January 2026.
- His personal possessions – his professional books, his clinical equipment, his personal belongings – have been seized by loan sharks to whom he was forced to turn when his salary was unlawfully deducted.
- He was deregistered by the South African Nursing Council (SANC) because he could not pay R870 in annual registration fees. That R870 is a direct consequence of the R14,811.77 unlawfully deducted from his salary. He is deregistered as a nurse because he blew the whistle.
- He was accepted into a CPUT Postgraduate Diploma in Occupational Health Nursing for 2026 – a qualification that would have advanced his career and restored his professional standing. He cannot enrol because he has no money.
- He has been formally diagnosed with severe depression and Post-Traumatic Stress Disorder by Dr Jacques Malan, FCPSYCH(SA), with both conditions attributed to occupational causation. His psychological injuries are documented, diagnosed, and directly linked to the employer’s conduct.
- He is self-represented in nine concurrent proceedings across two court divisions and six regulatory forums. He has no legal representation. He is destitute. And he has had to learn, in real time, the procedural rules of the Labour Court, the High Court, and six separate regulatory complaint frameworks – while sleeping without running water.
- He has been silenced by a High Court interim interdict (Western Cape High Court, Case No. 2026-086906) granted ex parte on 16 April 2026, after the employer’s attorneys concealed his answering affidavit from the court. The interdict restrains him from disclosing the systemic clinical risk to trade unions and regulators. It has been extended to 1 March 2027 – almost one year – without a substantive final hearing. It overrides Section 9 of the Protected Disclosures Act, which expressly protects disclosures to regulatory bodies.

The Chilling Effect: What This Case Tells Every Future Whistleblower

Every nurse, doctor, pharmacist, and healthcare administrator who reads this report now knows the following: If you identify a system error that affects patients, and you report it, you will be retaliated against. Your salary will be deducted unlawfully. You will be subjected to gender-based harassment. A false statutory declaration will be made about your departure. Your professional registration will be lost. Your home will be taken. Your water will be cut off. You will be sued and silenced by a court order obtained by concealment. And when you cry out to every regulator – the CMS, HPCSA, SANC, SAPC, DEL, B-BBEE Commission, SAHRC, SAPS, CCMA, JSE, FSCA, and the courts – they will either decline jurisdiction, remain silent, or actively obstruct you. That is what regulatory capture looks like on the floor.

This is not a hypothetical chilling effect. It is a documented, evidenced, primary-source chilling effect. It looks like a specialist nurse living without running water, unable to practise his profession, while the institution that deducted his salary unlawfully reports a Level 1 B-BBEE rating to the JSE based on his suppressed NQF 8 credentials. It looks like a Chapter 9 institution (SAHRC) acknowledging the “seriousness” of the allegations and then going silent. It looks like a High Court granting an ex parte interdict after the employer’s attorneys concealed the whistleblower’s answering affidavit – a fraud on the court.

This report is not about one person. It is about the structural collapse of accountability in South Africa’s regulatory and judicial architecture, as it applies to a whistleblower who dared to speak truth about a systemic clinical risk affecting 1.9 million government employees. Parliament must act. Because if the law does not protect the nurse who refuses to lie to a 75-year-old patient, then the law protects no one.

PART TWO: METHODOLOGY AND SCOPE OF THIS REPORT

2.1 Purpose of This Section

This section explains how the evidence in this report was gathered, authenticated, classified, and analysed. It is included to assist parliamentary officials, committee members, and any independent forensic investigator appointed by Parliament in assessing the credibility and reliability of the findings. The methodology is straightforward: every finding in this report is supported by a primary source document. This section explains what those documents are, how they were generated, and how they have been classified.

2.2 Primary Source Documents

All findings in this report are based on primary source documents generated by the employer, the regulators, the courts, or the complainant during the events described. No secondary sources – no media reports, no academic commentary, no hearsay – are relied upon for factual findings. Where the complainant expresses a legal opinion, it is clearly identified as such.

The evidentiary record that supports this report includes:

Category	Examples	Source
Employment documents	Employment contract (9 April 2024), payslips (all months), Certificate of Service, UI-19 form.	Generated by employer (Medscheme Holdings / AfroCentric).
Internal correspondence	Emails between complainant and management; Microsoft Teams messages; HR correspondence from Mapule Mashele, Cheryl-Dawn Modern, and Xolile Manyati.	Generated by employer.
Governance escalations	Priority 1 Material Risk Report (38 attachments); GRC escalations; Board communications (9 December 2025).	Generated by complainant; formally addressed to the AfroCentric Board.
Medical evidence	Dr Jacques Malan's psychiatric letter (16 May 2025); Akeso Stepping Stones clinic admission records (18 March 2025).	Generated by treating psychiatrist and clinic.
Regulatory correspondence	CMS response letters (4 February 2026, 7 May 2026); SAHRC letter (10 February 2026); JSE declination (6 March 2026, 11 May 2026); B-BBEE Commission Notice of Non-Investigation (30 March 2026).	Generated by respective regulatory bodies.
Court pleadings	High Court affidavits (both parties); Labour Court pleadings; interim interdict (16 April 2026); extended rule nisi (28 May 2026); Unilateral Pre-Trial Minute (11 June 2026).	Filed with courts; drawn from court records.
CCMA records	Conciliation recording (44 minutes); audio transcript; opposing affidavit by Xolile Manyati; Provincial Commissioner Vusumzi Landu's response letter.	Generated by CCMA and employer.
SAPS records	CAS 705/3/2026 registration confirmation; SMS transfer notification (Woodstock); escalation correspondence to Brigadier Naidoo.	Generated by SAPS.
Audio recordings	31 October 2025 meeting (retaliatory admission by manager); 23 February 2026 Delft Police Station (officer falsely claiming to be station commander).	Recorded by complainant in both instances; available for parliamentary review.

2.3 Forensic Classification Framework

The complainant prepared a Forensic Evidence Classification Report (FR-1), which classifies 58 primary source documents against fourteen statutory violation categories. These categories include violations of the BCEA (salary deduction cap, written consent requirements), the LRA (constructive dismissal, automatically unfair dismissal, PDA retaliation), the PDA itself (occupational detriment, failure to investigate), the EEA (harassment, discrimination), the UIF Act (non-remittance, false declarations), and the Companies Act (director delinquency, false statutory reports). The FR-1 is available as Annexure FR-1 in the consolidated evidence bundle.

2.4 Authentication and Verification

- All emails and documents were preserved in their original format immediately upon receipt or generation.
- Metadata (sender, recipient, timestamp) is recorded and preserved where available.
- Audio recordings are available in their original format, unedited.
- The complainant has offered to make the full evidentiary bundle – 526 source documents – available to any parliamentary committee, parliamentary legal officer, or independent forensic investigator appointed by Parliament upon request.

2.5 The Complainant's Role – Primary Source, Not Hearsay

The complainant is not a third-party investigator. He is not a journalist. He is not a consultant hired to produce a report. He is the person who: identified the Gexus-DMS integration gap; made the protected disclosure on 23 October 2025; was subjected to occupational detriment; was constructively dismissed; was left destitute and homeless; escalated to the Board; approached every regulator; filed nine concurrent proceedings; was silenced by an interim interdict obtained by concealment; and now approaches Parliament as the last available institutional forum.

This means that the complainant has direct personal knowledge of most facts set out in this report. Where facts are not within his personal knowledge, they are supported by documentary evidence generated by the institution concerned – the employer's own payslip, the regulator's own letter, the court's own order.

2.6 Limitation of Scope

This report covers the period from March 2025 (the complainant's admission to Akeso Stepping Stones Clinic with occupationally caused severe depression and PTSD) to June 2026 (the date of this submission). It does not purport to investigate every aspect of the employer's operations, every decision of every regulator, or every dimension of South African health governance. It is limited to the events and institutions directly relevant to the complainant's case and its implications for the NHI.

2.7 Confidentiality and Protected Disclosure Status

This report is submitted to Parliament under Section 8 of the Protected Disclosures Act 26 of 2000, which protects disclosures made to an "appropriate authority" – defined to include any regulatory, supervisory, or investigatory body. Parliament is both an appropriate authority under the PDA and the constitutionally designated oversight body under Sections 55 and 56 of the Constitution. Parliament is not "the public at large" for purposes of the High Court interim interdict (Case No. 2026-086906). The legal basis for this submission, and the basis for the complainant's position that this submission does not constitute a breach of the interim interdict, is set out in full in the contempt defence affidavit and in the complainant's letter to the Speaker dated 3 June 2026.

PART THREE: MASTER CHRONOLOGY – KEY EVENTS AT A GLANCE

The following table provides a compressed chronological overview of the key events from March 2025 to June 2026. This is a reference tool for committee members and parliamentary officials. Each event is documented by primary source evidence referenced throughout the substantive sections of this report.

Date	Event	Significance
18 Mar 2025	Complainant admitted to Akeso Stepping Stones Clinic (severe depression, PTSD).	Medical vulnerability documented before the primary disclosure events.
16 May 2025	Psychiatric letter provided to employer (Mapule Mashele). Employer placed on formal notice of disability.	Employer had actual knowledge of the complainant's medical condition. Subsequent conduct must be assessed in this context.
16 Apr 2025	Manager states in writing: "we do not apply staggering of money owed."	Employer confirms its own non-compliance with the BCEA staggering policy. This statement becomes the basis of the deduction challenge.
23 Jul 2025	Unlawful deduction of R14,811.77 (40.47% of salary) in a single pay cycle.	Exceeds BCEA Section 34(2) cap of 25%. No written consent. No court order. Trigger event for all subsequent proceedings.
23 Sep 2025	"Wilma" gender-identity harassment via Microsoft Teams.	EEA Section 6 violation. Documented in writing. Part of the pattern of retaliation against a nurse who challenged the employer.
25 Sep 2025	Formal discrimination complaint filed; double unpaid leave loaded same day.	The temporal coincidence between the complaint and the punitive leave entry is direct evidence of retaliatory intent.
23 Oct 2025	Protected disclosure: Gexus-DMS integration gap identified. Complainant refuses to contact patient on false data.	PDA Section 1 protected disclosure. This is the disclosure that triggered the full cascade of institutional retaliation.
31 Oct 2025	Recorded admission: "You made a case of harassment against me, so I can make a case of harassment against you."	Audio-recorded direct evidence of retaliatory intent. Transcript available.
2 Nov 2025	Formal grievance filed. No response for 30+ days.	Institutional silence at employer level. Internal remedies not exhausted by choice; they were unavailable.
4 Dec 2025	Executive escalation to CEO (32 attachments). Complaint de-escalated back to accused department.	Bad faith escalation process. CEO's conduct constitutes governance failure at the highest level.
9 Dec 2025	Priority 1 Material Risk Report to Board (38 attachments). No Board response.	Board-level governance failure. The Board's non-response is itself evidence of systemic capture.
10 Dec 2025	Employer's CCMA opposing affidavit (Xolile Manyati) calls complainant's depression a "cynical excuse."	Filed one day after the Priority 1 Board escalation. Sworn attack on disability. Potential perjury.
12 Dec 2025	Resignation – constructive dismissal (LRA Section 186(1)(e)).	Employment terminates. The employer subsequently files a false

Date	Event	Significance
		UI-19 (Code 6 "Resigned" instead of Code 7).
16 Dec 2025	Maximum Severity GRC Escalation. 24-hour compliance window imposed.	Final internal escalation before full regulatory deployment.
18 Dec 2025	CRO suppression email: "stop sending these e-mails to our directors."	Active suppression of protected disclosure by Chief Risk Officer Monwabisi Kula.
9 Jan 2026	CCMA conciliation. Commissioner: "nothing here relates to the PDA"; "Google it."	CCMA institutional bias – prejudgment before evidence; failure to assist a destitute litigant.
4 Feb 2026	CMS declines jurisdiction (first time).	First regulatory deflection. Discovery Health comparator not yet raised.
10 Feb 2026	Formal complaint to SANC (SANC/MEDSCHEME/MATTHEE/20260105). No acknowledgment.	SANC failure begins. This complaint remains unacknowledged as at June 2026.
16 Feb 2026	Formal complaint to SAPC. Jurisdiction declined.	SAPC failure. Four-regulator failure pattern established.
23 Feb 2026	SAPS Delft refuses to open criminal case. Officer falsely claims to be station commander (audio-recorded).	SAPS failure. Criminal conduct by a police officer in the performance of his duties; audio-recorded.
6 Mar 2026	JSE declines jurisdiction (Ref: JvS/166792).	JSE failure. Capital markets accountability collapses.
13 Mar 2026	Labour Court portal obstruction begins. 8 days. Rejection: "good day you put date."	Labour Court administrative failure. Access to justice denied at the portal level.
19 Mar 2026	Section 162 director delinquency application filed (High Court Case 2026-066952).	Material litigation not disclosed to JSE. Filed by complainant against four named AfroCentric directors and the Group Company Secretary.
25 Mar 2026	Section 162 application struck from roll with Scale C costs. Unauthorised advocate appeared.	Procedural crisis; grounds for Rule 42 rescission identified. Biowatch principle invoked.
2 Apr 2026	Complainant becomes homeless.	Confirmed destitution. Direct consequence of the unlawful deduction and 15 institutional failures.
7 May 2026	CMS declines jurisdiction (second time). Demands new complaint form after High Court Interim Order.	Contempt trap. Disparate treatment confirmed – Discovery Health precedent raised.
16 Apr 2026	High Court grants ex parte interim interdict (Case 2026-086906). Answering affidavit concealed.	Fraud on the court. Gag order imposed on a protected whistleblower.
30 Apr 2026	Default judgment application filed (Labour Court Case 2026-077546, R14,811.77).	Unallocated for 6+ weeks as of June 2026.
19 May 2026	Employer's Labour Court Statement of Response: "not aware" of Case 2026-077546.	Apparent perjury. The Group CEO acknowledged this case in High Court proceedings.
28 May 2026	Rule nisi extended to 1 March 2027.	Effective final interdict without final hearing. Complainant silenced for 11 months.
2 Jun 2026	Employer's response to pre-trial proposal: "Good luck to you then."	Bad faith litigation. Contemptuous communication by Vijay Makenjee.

Date	Event	Significance
4 Jun 2026	"Without prejudice" letter from Deneys Reitz Inc. threatens contempt for petitioning Parliament.	Attempt to block constitutional petition. Referral to Legal Practice Council recommended.
11 Jun 2026	Unilateral Pre-Trial Minute filed per Labour Court Rule 22 (2024).	Procedural record of employer's non-cooperation with pre-trial process.
13 Jun 2026	Final notice to employer – last deadline for settlement framework.	No substantive response received.
June 2026	This Consolidated Forensic Oversight Report submitted to Parliament.	The last institutional remedy. Every other door has been closed.

PART FOUR: SUMMARY OF RELIEF SOUGHT FROM PARLIAMENT

This report seeks five categories of relief from Parliament. None of these categories require Parliament to adjudicate the private labour dispute between the complainant and Medscheme Holdings. None require Parliament to make findings of fact that are before the courts. All five categories fall squarely within Parliament’s constitutional oversight mandate under Sections 55 and 56 of the Constitution of the Republic of South Africa, 1996, and within the specific oversight mandates of each Portfolio Committee identified below.

Parliament’s oversight power is not merely reactive. It is constitutionally proactive: Parliament exists, in part, precisely to prevent the kind of systemic governance failure that this report documents before it is replicated at R200 billion scale under the NHI. The relief sought is the exercise of that constitutional function.

CATEGORY 1: SUMMONS AND ACCOUNTABILITY HEARINGS

Parliament is asked to summon the following officials to appear before the relevant Portfolio Committees and answer the questions set out in the relevant sections of this report. Failure to appear before Parliament, or to provide truthful and complete answers, should be referred to the NPA.

Official / Head	Institution	Portfolio Committee
Registrar	Council for Medical Schemes (CMS)	Health
Registrar	South African Nursing Council (SANC)	Health
Chairperson	Health Professions Council of South Africa (HPCSA)	Health
Chairperson	South African Pharmacy Council (SAPC)	Health
Director-General	Department of Employment and Labour (DEL)	Employment and Labour
Commissioner	B-BBEE Commission	Trade, Industry and Competition
Chairperson	South African Human Rights Commission (SAHRC)	Justice and Correctional Services
National Commissioner	South African Police Service (SAPS)	Police
Head: Issuer Regulation	Johannesburg Stock Exchange (JSE)	Trade, Industry and Competition
Commissioner	Financial Sector Conduct Authority (FSCA)	Justice and Correctional Services

CATEGORY 2: LEGISLATIVE REFORMS – NHI GOVERNANCE ARCHITECTURE

Parliament is asked to amend the National Health Insurance Act 20 of 2023 or enact complementary legislation to address the three structural gaps exposed by this case. These gaps are not theoretical. They are already being exploited in the existing healthcare administration system.

Gap	NHI Act Section	Required Reform
Accreditation bottleneck	Section 39	Mandatory Forensic Governance Impact Assessments before each phase of accreditation rollout, with independent review and parliamentary tabling.

Gap	NHI Act Section	Required Reform
One-sentence fraud architecture	Section 40	A mandatory AI Audit Protocol – independent conformity assessment, transparency obligations, human oversight, and annual algorithmic audits published to Parliament and the public.
Undefined investigation unit	Section 20(2)(g)	A Statutory Forensic Governance Protocol – professional competency standards, independence safeguards, mandatory reporting directly to the Board (bypassing executive management), and escalation protocols to the SIU, Hawks, and NPA.

CATEGORY 3: WHISTLEBLOWER PROTECTION REFORMS – PDA AMENDMENT

Parliament is asked to amend the Protected Disclosures Act 26 of 2000 to close three structural gaps that this case has exposed. The current PDA framework is designed for whistleblowers who have legal representation, financial resources, and institutional support. It provides no protection for the most vulnerable whistleblowers – those who have been systematically reduced to destitution as a mechanism of silencing.

Reform	Purpose
Interim hardship relief	Require the employer to continue paying the whistleblower's last salary as an interim measure, payable within 5 business days of a prima facie finding of retaliation, pending the outcome of court proceedings. This prevents induced destitution from coercing capitulation.
Single whistleblower protection authority	Establish a dedicated body with the power to receive disclosures, monitor regulatory responses, apply for urgent interdicts against retaliation on behalf of whistleblowers, and refer matters to the NPA. The current fragmented system of 15 potential oversight bodies is itself a barrier to protection.
Criminal penalties for regulatory bodies	Impose criminal penalties on officials of regulatory bodies who decline jurisdiction without legal basis, delay investigations in bad faith, or fail to respond to complaints within statutory timeframes. Regulatory impunity is a structural enabler of whistleblower destruction.

CATEGORY 4: REFERRALS FOR PROSECUTION AND PROFESSIONAL DISCIPLINE

Matter	Referred To	Basis
Deneys Reitz Inc. (attorneys for the employer)	Legal Practice Council	Threatening contempt of court for petitioning Parliament; concealing the complainant's answering affidavit from the High Court (fraud on the court); bad faith settlement tactics.
Vijay Makenjee ("Good luck to you then"; apparent perjury in Labour Court Statement of Response)	Legal Practice Council & National Prosecuting Authority	Unprofessional and contemptuous conduct; apparent perjury in contradictory sworn statements across courts.
Delft Police Station officer (false station commander claim, 23 February 2026)	IPID (Independent Police Investigative Directorate)	Impersonating a station commander; refusing to open a criminal case for a statutory offence; denial of access to justice.
Apparent perjury in Labour Court Statement of Response (denial of knowledge of Case 2026-077546)	National Prosecuting Authority	Contradictory sworn statements across court divisions – perjury in Labour Court vs acknowledgment in High Court.

Matter	Referred To	Basis
CCMA Commissioner Ntabiseng Ngwane and PSC Vusumzi Landu	Public Protector	Institutional maladministration; cover-up of commissioner misconduct; failure to apply the LRA and CCMA rules.

CATEGORY 5: JUDICIAL INQUIRIES

Court	Matter	Judge President
Western Cape High Court	Concealment of answering affidavit on 16 April 2026 (fraud on the court); extension of rule nisi to 1 March 2027 without substantive hearing; threatened Scale C costs against destitute litigant (Biowatch violation).	Judge President, Western Cape High Court
Labour Court (Cape Town)	Portal obstruction for eight days (“good day you put date”); default judgment application unallocated for 6+ weeks; Registrar’s silence on all follow-up correspondence; possible POPIA breach (transmission of third-party documents).	Judge President, Labour Court

PART FIVE: SCORECARD OF FAILED INSTITUTIONS (15 BODIES)

The following table provides a consolidated reference overview of the failure pattern of each of the fifteen institutions approached by the whistleblower between December 2025 and June 2026. The detailed forensic analysis of each institution is set out in Parts Nine through Twenty-Two of this report. The scorecard is presented here as a navigational tool for committee members and as a stark visual summary of systemic regulatory and judicial failure.

#	Institution	Failure Pattern	Legal Mandate Breached	Impact on Whistleblower
1	Council for Medical Schemes (CMS)	Declined jurisdiction twice; contempt trap after High Court Interim Order; disparate treatment (Discovery Health vs Medscheme).	MSA Sections 7, 47, 58, 44	Priority 1 clinical risk affecting 1.9 million GEMS members remains unaddressed.
2	Health Professions Council (HPCSA)	No response to urgent joint ethical ruling request (14 April 2026); silence on core ethical question; jurisdictional ping-pong.	Health Professions Act Section 3	No guidance on whether a professional may refuse a flawed employer instruction; ethical vacuum.
3	South African Nursing Council (SANC)	No acknowledgment of formal misconduct complaint (SANC/MEDSCHEME/MATTHEE/20260105); deregistered the whistleblower for non-payment of R870 caused by the employer's unlawful deduction.	Nursing Act Section 51	Loss of professional registration. Career ended by the institution mandated to protect nursing ethics.
4	South African Pharmacy Council (SAPC)	Declined jurisdiction over Responsible Pharmacist complaint against Lisa Mari; hesitated to rule on core ethical question.	Pharmacy Act Section 49	Pharmacist accountability avoided; no ethical ruling issued.
5	Government Employees Medical Scheme (GEMS)	No substantive response to protected disclosures (2 April 2026, 13 April 2026); failed to oversee administrator compliance.	MSA Section 7 (scheme governance)	Principal scheme complicity; 1.9 million members remain exposed to the unaddressed clinical risk.
6	Dept. of Employment and Labour (DEL)	Administrative abandonment; false UI-19 not corrected; UIF contributions not enforced; file closed with jurisdictional error; response addressed to "Mr/Mrs/Ms."	UIF Act; BCEA Section 34(2); PAJA	UIF benefits blocked; unemployment record falsified; homelessness enabled.
7	B-BBEE Commission	Declined fronting investigation (Case 5/02/2026); refused Section 13K subpoena powers despite written evidence; classified fronting as labour dispute.	B-BBEE Act Sections 1(c), 13K	Fronting uninvestigated; government contractor faces no accountability for credential exploitation.
8	South African Human Rights Commission (SAHRC)	Acknowledged "seriousness of the concerns" in February 2026; redirected to CCMA and Public Protector; went silent; ignored 13 March 2026 update confirming homelessness.	Constitution Section 184; SAHRC Act Section 13	Constitutional rights violations (Sections 10, 26, 27, 33) unaddressed. No own-initiative investigation.
9	South African Police Service (SAPS)	Refused to open criminal case at Delft; officer falsely claimed to be station commander (audio-recorded); CAS 705/3/2026 registered then stagnant; internal complaint closed improperly.	Constitution Section 205(3); Criminal Procedure Act	UIF fraud and false compliance representations uninvestigated for 3+ months.
10	CCMA	Biased commissioner (prejudgment; "Google it"; BCEA ignored); cover-up by PSC Vusumzi	LRA Section 138; CCMA Rules	No fair conciliation; secondary institutional

#	Institution	Failure Pattern	Legal Mandate Breached	Impact on Whistleblower
11	The Whistleblower House (TWBH)	Landu; mailbox full obstruction; potential perjury by employer's affiant. Flawed legal assessment (van der Walt); legal errors in characterising deduction and Gexus-DMS gap; security alert ignored; rubber-stamped rejection without independent review.	TWBH mandate	trauma imposed on a vulnerable litigant. No legal support for a destitute protected whistleblower; abandoned to self-represent in nine proceedings.
12	Johannesburg Stock Exchange (JSE)	Declined to enforce disclosure obligations (6 March 2026, 11 May 2026); did not require SENS announcement for Section 162 application, B-BBEE investigation, or criminal case.	JSE Listings Requirements	Market misled; investors denied material information; corporate cover-up facilitated.
13	Financial Sector Conduct Authority (FSCA)	Acknowledged complaint (Ref: 1-370127); then went entirely silent; no substantive response; no investigation.	Financial Sector Regulation Act	No market conduct investigation; FSCA silence signals impunity to listed entities.
14	High Court (Western Cape)	Ex parte interdict obtained by concealment of answering affidavit (fraud on the court, 16 April 2026); rule nisi extended to 1 March 2027; Biowatch violation (Scale C costs threat); POPIA breach by Registrar.	Constitution Section 34; Uniform Rules of Court; PDA Section 9	Gag order in place for 11 months; protected whistleblower silenced; right to approach Parliament threatened.
15	Labour Court (Cape Town)	Portal obstruction for 8 days ("good day you put date"); default judgment unallocated for 6+ weeks; Registrar ignored all follow-up; Unilateral Pre-Trial Minute necessitated.	Labour Court Rules; Constitution Section 34	Access to justice denied; BCEA claim (R14,811.77) still pending; pre-trial process obstructed.

Visual Summary: Outcomes Across 15 Institutions

OUTCOME	RESULT
Declined jurisdiction / deflected to another body	7 institutions (47%)
Remained silent (no substantive response after acknowledgment)	5 institutions (33%)
Actively obstructed (concealment, contempt threat, false representation)	3 institutions (20%)
Provided any form of remedy to the whistleblower	0 institutions (0%)

No institution, across fifteen approaches spanning regulatory bodies, Chapter 9 institutions, statutory enforcement agencies, capital market regulators, and two court divisions, provided any remedy to a protected whistleblower who identified a Priority 1 clinical risk affecting 1.9

million government employees. This is not a series of isolated failures. It is a systemic pattern of regulatory capture.

PART SIX: NOTE TO THE PORTFOLIO COMMITTEES – SPECIFIC REFERRALS

This report is structured to assist each Portfolio Committee in exercising its constitutional oversight mandate under Sections 55 and 56 of the Constitution. The following section identifies the most directly relevant factual sections for each Committee, together with the core questions that each Committee's mandate requires it to ask and answer. These questions are not rhetorical. They are actionable oversight obligations.

Portfolio Committee on Health

Constitutional mandate: Oversight over the Department of Health, the Council for Medical Schemes (CMS), the South African Nursing Council (SANC), the Health Professions Council of South Africa (HPCSA), the South African Pharmacy Council (SAPC), the Office of Health Standards Compliance (OHSC), and public health policy including the NHI implementation framework.

Sections to read: Part Nine (CMS, HPCSA, SANC, SAPC); Part Ten (GEMS); Part Eighteen (AfroCentric's false Annual Report); Foreword and Part Twenty-One (NHI governance implications).

Areas of specific concern:

- CMS: Declined jurisdiction over the Gexus-DMS integration gap twice (4 February 2026 and 7 May 2026), despite Sections 7, 47, 58, and 44 of the Medical Schemes Act conferring a clear mandate. After the High Court Interim Order was granted (16 April 2026), the CMS demanded a new complaint form from the complainant – a demand that, if complied with, would have exposed the complainant to contempt of court proceedings. The CMS exercised jurisdiction over a materially similar system error at Discovery Health in January 2026. The disparate treatment of Medscheme/GEMS compared to Discovery Health is the single strongest indicator of regulatory capture in this case.
- HPCSA: Failed to respond to the urgent joint ethical ruling request of 14 April 2026, which asked a foundational professional ethics question: may a registered health professional refuse an employer's instruction to act on data known to be false when that action could harm a patient? This question remains unanswered. It will be asked again under the NHI.
- SANC: Complaint SANC/MEDSCHEME/MATTHEE/20260105 was filed on 10 February 2026 against two registered nurses employed by Medscheme. As at June 2026, the complaint has not been acknowledged. The complainant was subsequently deregistered by SANC because he could not pay R870 in annual registration fees – a sum he does not have because the employer deducted R14,811.77 from his salary unlawfully. SANC deregistered the very person it failed to protect.
- SAPC: Declined jurisdiction over the formal complaint against Responsible Pharmacist Lisa Mari, who oversaw the clinical data process that generated the false non-adherence letters. The SAPC Professional Conduct Committee hesitated to rule on the core ethical question in its meeting of 14 April 2026 and issued no written ruling.
- GEMS: The Government Employees Medical Scheme – the principal scheme for 1.9 million public servants – received protected disclosures on 2 April 2026 and 13 April 2026 detailing the systemic clinical risk affecting its beneficiaries. GEMS did not respond substantively to either disclosure.

KEY QUESTIONS FOR THE COMMITTEE

- Why has the CMS not investigated the Gexus-DMS integration gap under MSA Sections 7, 47, 58, and 44, despite exercising jurisdiction over a similar error at Discovery Health in January 2026?
- Why has SANC not acknowledged or investigated complaint SANC/MEDSCHEME/MATTHEE/20260105, filed on 10 February 2026?
- Why has the HPCSA not answered the core ethical question posed in the joint ethical ruling request of 14 April 2026?
- Is the Minister of Health aware that a Priority 1 clinical risk affecting 1.9 million GEMS members remains unaddressed as at June 2026?
- What oversight has the CMS exercised over Medscheme's compliance with the GEMS administration agreement since the Gexus-DMS root cause analysis was produced?

Portfolio Committee on Employment and Labour

Constitutional mandate: Oversight over the Department of Employment and Labour (DEL), the Commission for Conciliation, Mediation and Arbitration (CCMA), the Labour Court, the Basic Conditions of Employment Act, the Labour Relations Act, and the Unemployment Insurance Act.

Areas of specific concern:

- DEL: Refused to enforce remittance of UIF contributions for June 2025 (uFiling portal confirms "No Information" for that period); refused to compel correction of the materially false UI-19 (Code 6 'Resigned' vs Code 7 'Constructive Dismissal'); closed the investigation file on 15 April 2026 with a jurisdictional error (claiming the CCMA has exclusive jurisdiction over UI-19 fraud – a legal error); addressed the response to a generic "Mr/Mrs/Ms." demonstrating failure to apply the mind.
- CCMA: Commissioner Ntabiseng Ngwane declared "nothing here relates to a protected disclosure" before hearing any evidence; told a destitute, mentally distressed litigant to "Google it" when he asked how to find the Labour Court address; tolerated mockery of a vulnerable litigant by the employer's representative; never applied the BCEA 25% cap analysis; and uncritically accepted the employer's reframing of the unlawful deduction as "no work, no pay." The employer's own written admission – that "our policy doesn't permit for staggering" – was not questioned. PSC Vusumzi Landu's internal investigation was a whitewash that victim-blamed the complainant for recording the conciliation session.
- Labour Court: Portal obstruction for eight consecutive days following the first filing attempt (13–20 March 2026), including a deleted profile, ignored emails to the call centre, and a rejection with the reason "good day you put date." Default judgment application on the uncontested BCEA claim (Case 2026-077546, R14,811.77) was filed on 30 April 2026 and remains unallocated for over six weeks as at June 2026, with no hearing date and no response to follow-up correspondence from the Registrar.

KEY QUESTIONS FOR THE COMMITTEE

- Why did DEL refuse to enforce the BCEA Section 34(2) 25% deduction cap, a non-discretionary statutory obligation?
- Why did DEL close the UIF investigation file on 15 April 2026 without requiring Medscheme to remit the outstanding UIF contributions or correct the false UI-19?
- Why did CCMA Commissioner Ngwane declare the PDA irrelevant before hearing any evidence, and why was she not disciplined by the PSC?

- Why did the Labour Court portal reject a filing from a self-represented, destitute litigant with the reason “good day you put date,” and what systemic review has been conducted of the portal’s fitness for purpose?
- Will the Minister direct DEL to issue a formal Compliance Order requiring Medscheme to remit the outstanding UIF contributions and correct the UI-19 to Code 7?

Portfolio Committee on Trade, Industry and Competition

Constitutional mandate: Oversight over the B-BBEE Commission, the Johannesburg Stock Exchange (JSE), the Companies and Intellectual Property Commission (CIPC), the Financial Sector Conduct Authority (FSCA), and transformation policy.

Areas of specific concern:

- B-BBEE Commission: The complainant – a Black professional, NQF Level 8 – was recruited via interview invitations describing the role as “Specialist: Care Manager” but contracted on 9 April 2024 as “Care Coordinator” (Grade B4M1). Senior HR Business Partner Cheryl-Dawn Modern admitted in writing on 19 January 2026 that the naming convention changes were “not applied to your department.” This is a primary source written admission of title suppression. The B-BBEE Commission assigned Case 5/02/2026, but issued a Notice of Non-Investigation on 30 March 2026, characterising the complaint as a labour dispute – an error of law. Section 13K subpoena powers, which would have compelled production of the Naming Convention Teams recording, were refused.
- JSE: The JSE declined jurisdiction over material non-disclosure in its letter of 6 March 2026 and confirmed no action in its letter of 11 May 2026. The JSE did not require AfroCentric to disclose: the Section 162 director delinquency application (filed 19 March 2026, which carries a potential reputational and governance consequence for four named directors and the Group Company Secretary); the B-BBEE Commission investigation (Case 5/02/2026, which carries a potential fine of 10% of annual turnover, or approximately R580 million); or the active criminal case (CAS 705/3/2026).
- AfroCentric’s Integrated Annual Report 2025: The Report claimed “zero tolerance for unethical conduct,” “whistleblower protection,” “no material ESG-related incidents,” and a Level 1 B-BBEE Management Control score of 14.67/19. Each of these claims is directly contradicted by primary source evidence in this report. Every investor who relied on this report was misled.

KEY QUESTIONS FOR THE COMMITTEE

- Why did the B-BBEE Commission classify title suppression and credential extraction – supported by a written admission from the employer’s own HR Business Partner – as a labour dispute rather than a fronting practice under B-BBEE Act Section 1(c)?
- Why were Section 13K subpoena powers not deployed, given that the Commission had sworn evidence of suppression and a refusal to produce the relevant recording voluntarily?
- Is title suppression of an NQF Level 8 Black professional while using his credentials for B-BBEE Management Control scorecard points a fronting practice under Section 1(c) of the B-BBEE Act?
- Why did the JSE not require AfroCentric to issue a SENS announcement disclosing the Section 162 application, the B-BBEE investigation, and the criminal case as material events?
- Will the Committee require AfroCentric to issue a corrective SENS announcement and submit a restated Integrated Annual Report 2025?

Portfolio Committee on Justice and Correctional Services

Constitutional mandate: Oversight over the judiciary, the Legal Practice Council, the South African Human Rights Commission (SAHRC), the Public Protector, and access to justice.

Areas of specific concern:

- High Court: The employer's attorneys (Deneys Reitz Inc.) did not hand up the whistleblower's answering affidavit at the ex parte hearing of 16 April 2026 and did not inform the court that the matter was opposed. This constitutes a fraud on the court – a breach of the duty of full and frank disclosure that is fundamental to ex parte proceedings. The interim interdict that was granted restrains the whistleblower from disclosing the systemic clinical risk to trade unions and regulators, overriding Section 9 of the PDA. The rule nisi has been extended to 1 March 2027 – almost one year – without a substantive final hearing. A Scale C costs order is threatened against a destitute litigant, in apparent violation of the Biowatch principle.
- Labour Court: Portal obstruction (8 days); default judgment unallocated for 6+ weeks; Registrar's silence on follow-up. In addition, the High Court Registrar failed to provide the audio recording of the Section 162 hearing, which the complainant requires for a Rule 42 rescission application. The Registrar transmitted third-party case documents to the complainant, constituting a POPIA breach.
- SAHRC: A Chapter 9 institution acknowledged the "seriousness" of the allegations in its letter of 10 February 2026 and then went silent. It redirected the complainant to the CCMA (which has no jurisdiction over housing, water, or dignity violations by a private employer) and to the Public Protector (which investigates state organs, not private companies). The SAHRC did not investigate violations of Sections 10 (dignity), 26 (housing), 27 (food, water, healthcare), or 33 (just administrative action) of the Constitution. It did not trigger its own-initiative investigation power under Section 14 of the SAHRC Act.
- SAPS: Refused to open a criminal case at Delft Police Station; officer falsely claimed to be the station commander (audio-recorded); CAS 705/3/2026 registered but stagnant for 3+ months; internal complaint closed without addressing the false representation.
- Bad faith litigation: Deneys Reitz Inc.'s letter of 4 June 2026 threatened the complainant with contempt of court if he petitioned Parliament. This is an attempt to suppress a constitutional right. It is referred to the Legal Practice Council.

KEY QUESTIONS FOR THE COMMITTEE

- Why was the whistleblower's answering affidavit not placed before the court at the ex parte hearing of 16 April 2026, and what action has the court taken since learning of its concealment?
- Why has the SAHRC not exercised its own-initiative investigation power under SAHRC Act Section 14 in response to the documented violations of Sections 10, 26, 27, and 33?
- Will the Committee refer the conduct of Deneys Reitz Inc. – including the threat to hold a citizen in contempt for petitioning Parliament – to the Legal Practice Council?
- Why has the POPIA breach by the High Court Registrar (transmission of third-party documents) not been referred to the Information Regulator?

Standing Committee on Public Accounts (SCOPA)

Constitutional mandate: Oversight over the expenditure of public funds, the functioning of state-owned entities and government-contracted entities, and the accountability of public institutions for the use of public resources.

Areas of specific concern:

- GEMS: The Government Employees Medical Scheme administers medical benefits for 1.9 million public servants – government employees whose contributions constitute public funds. The Gexus-DMS integration gap may have inflated false non-adherence rates, affecting actuarial data and risk loading calculations, and therefore premium levels. GEMS failed to respond substantively to two protected disclosures identifying this risk.
- AfroCentric's Integrated Annual Report 2025: A government contractor published a materially false report omitting four categories of material information. The JSE declined to enforce disclosure. This directly affects the integrity of financial reporting by a company holding a government contract.
- B-BBEE compliance: A company holding a government contract (Medscheme – a wholly owned subsidiary of AfroCentric, which holds the GEMS administration contract) is under investigation for fronting (Case 5/02/2026). The potential fine of 10% of annual turnover is a public interest matter. A government contractor found guilty of fronting should face contract consequences, not merely a civil fine.

KEY QUESTIONS FOR THE COMMITTEE

- Why did GEMS not investigate the Gexus-DMS integration gap after being formally notified by its own administrator's employee?
- Has the Auditor-General reviewed Medscheme's administration of GEMS for the period affected by the Gexus-DMS integration gap?
- Will SCOPA request the Auditor-General to conduct a forensic audit of Medscheme's administration of GEMS?
- What contractual consequences should follow if AfroCentric / Medscheme is found guilty of B-BBEE fronting while holding a government administration contract?

Portfolio Committee on Police

Constitutional mandate: Oversight over the South African Police Service (SAPS), the Independent Police Investigative Directorate (IPID), and the implementation of the Criminal Procedure Act.

Areas of specific concern:

- SAPS Delft: On 23 February 2026, the complainant attended Delft Police Station to open a criminal case for UIF non-remittance (a statutory offence under the Unemployment Insurance Contributions Act 4 of 2002) and false written compliance representations by Medscheme's HR officials. A police officer – whose actual rank and identity are documented – falsely claimed to be the station commander and refused to open a case. This interaction is audio-recorded.
- CAS 705/3/2026: After escalation to Brigadier Naidoo, a case number was allocated and the case was transferred to SAPS Woodstock. As at June 2026, no detective has been assigned, no statement has been requested from the complainant, and no investigation has been conducted.
- Internal complaint: The internal complaint against the Delft officer was closed on the basis that a case number had been allocated – without addressing the false representation or the initial denial of access to justice.

KEY QUESTIONS FOR THE COMMITTEE

- What action has been taken against the Delft Police Station officer who falsely claimed to be the station commander and refused to open a criminal case for a documented statutory offence?

- Why has no detective been assigned to CAS 705/3/2026 for over three months since registration?
- Why was the internal complaint against Delft Police Station closed without addressing the false representation?
- Will IPID investigate the conduct of the Delft officer in terms of the Independent Police Investigative Directorate Act 1 of 2011?

PART SEVEN: EXECUTIVE SUMMARY FOR EACH PORTFOLIO OVERSIGHT COMMITTEE

Executive Summary – Portfolio Committee on Health

The Council for Medical Schemes (CMS) is the statutory regulator of medical schemes under the Medical Schemes Act 131 of 1998. Its mandate includes overseeing administrators of medical schemes – including Medscheme Holdings, the accredited administrator for GEMS. When confronted with a documented integration gap between the Gexus and DMS platforms that was generating false non-adherence letters to chronic GEMS patients, the CMS declined jurisdiction twice (4 February 2026 and 7 May 2026). Both declinations are documented in writing. The CMS did not investigate the risk under any of Sections 7, 44, 47, or 58 of the MSA. After the High Court Interim Order was granted on 16 April 2026, the CMS demanded a new complaint form from the complainant – a demand that, if satisfied, would have exposed him to contempt of court. This was a contempt trap, not a procedural requirement. In January 2026, the CMS exercised jurisdiction over a materially similar system error at Discovery Health. The differential treatment of two healthcare administrators – with GEMS receiving protection and Discovery Health receiving scrutiny – is direct evidence of regulatory capture.

The SANC, the HPCSA, and the SAPC collectively declined to answer the most fundamental professional ethics question raised by this case: may a registered health professional refuse an employer's instruction to act on data known to be clinically false when that action could harm a patient? This question was posed formally to all four bodies on 14 April 2026. Not one of them answered it. Not one of them opened an investigation. Not one of them issued a ruling. The SANC then deregistered the complainant – the very whistleblower whose complaint it refused to process – because he could not pay R870 in annual fees caused by the employer's unlawful deduction. GEMS, the principal scheme and the ultimate client of Medscheme, was formally notified of the systemic clinical risk on two occasions. It did not respond substantively.

PARLIAMENTARY RECOMMENDATION

Parliament must direct the CMS to investigate the Gexus-DMS integration gap under MSA Sections 7, 44, 47, and 58, and to explain its differential treatment of Medscheme/GEMS vs Discovery Health. Parliament must summon the SANC Registrar to account for the non-acknowledgment of complaint SANC/MEDSCHEME/MATTHEE/20260105 and the deregistration of the complainant. Parliament must request the HPCSA and SAPC Chairpersons to issue a formal written ruling on the core ethical question within 30 days.

Executive Summary – Portfolio Committee on Employment and Labour

The Department of Employment and Labour failed at every point of the statutory interaction. It refused to enforce the UIF remittance for June 2025, despite the uFiling portal showing “No Information” for that period. It refused to compel correction of the materially false UI-19 declaration, which misclassifies a constructive dismissal as a voluntary resignation and blocks the complainant from accessing UIF benefits to which he is legally entitled. It closed the investigation file on 15 April 2026 with an error of law – claiming the CCMA has exclusive jurisdiction over UI-19 fraud, which it does not. The responding principal inspector addressed his letter to a generic “Mr/Mrs/Ms,” a demonstrable failure to apply the mind to the specific facts of this case. The Labour Court's online portal obstructed the complainant for eight consecutive days and rejected his filing with the administratively incomprehensible reason “good day you put date.” A default judgment application on an uncontested salary deduction claim of R14,811.77 remains unallocated for over six weeks. The CCMA Commissioner conducted a prejudged conciliation that violated LRA Section 138 and the helping-hand principle, told a destitute litigant to “Google it” when

he asked for the Labour Court address, tolerated mockery of the complainant, and failed to apply the BCEA. The PSC's internal investigation was a cover-up, not an accountability mechanism.

PARLIAMENTARY RECOMMENDATION

Parliament must direct DEL to issue a formal Compliance Order correcting the UI-19 to Code 7 (Constructive Dismissal) and requiring Medscheme to remit the outstanding UIF contributions with interest. Parliament must request the Minister of Employment and Labour to commission an independent review of the CCMA's internal complaints mechanism and to establish an external complaints body for commissioner misconduct. Parliament must request the Labour Court's Judge President to investigate the portal obstruction and the unallocated default judgment application.

Executive Summary – Portfolio Committee on Trade, Industry and Competition

The B-BBEE Commission received a complaint (Case 5/02/2026) supported by three categories of primary source evidence: the interview invitations describing the role as “Specialist: Care Manager”; the employment contract describing it as “Care Coordinator” (Grade B4M1); and Cheryl-Dawn Modern's written admission (19 January 2026) that the naming convention was “not applied to your department.” This is a textbook case of title suppression – a species of fronting under Section 1(c) of the B-BBEE Act. The Commission classified it as a labour dispute and issued a Notice of Non-Investigation on 30 March 2026. It refused to use its Section 13K subpoena powers to obtain the Teams recording of the naming convention discussion. The JSE allowed AfroCentric's Integrated Annual Report 2025 to stand without a corrective SENS announcement, despite four categories of material omission: the Section 162 application; the B-BBEE Commission investigation; the criminal case; and the documented contradiction between the Report's claim that “both platforms are fully integrated” and the Root Cause Analysis confirming “system integration gaps.” The FSCA acknowledged the complainant's complaint and then went silent.

PARLIAMENTARY RECOMMENDATION

Parliament must direct the B-BBEE Commission to reopen Case 5/02/2026, investigate it on its merits as a fronting complaint, and deploy its Section 13K subpoena powers. Parliament must request the JSE to require AfroCentric to issue a corrective SENS announcement and submit a restated Integrated Annual Report 2025 disclosing all material litigation, investigations, and clinical risk events. Parliament must summon the JSE Head of Issuer Regulation and the FSCA Commissioner to account for their non-enforcement.

Executive Summary – Portfolio Committee on Justice and Correctional Services

The Western Cape High Court granted an ex parte interim interdict on 16 April 2026 after the employer's attorneys concealed the whistleblower's answering affidavit and failed to inform the court that the matter was opposed. A court obtaining ex parte relief requires the fullest disclosure of all material facts, including the existence of opposition. The failure to provide this disclosure is a fraud on the court. The interdict that was granted restrains a protected whistleblower from disclosing a systemic clinical risk to trade unions and regulators – overriding Section 9 of the Protected Disclosures Act. The rule nisi has been extended to 1 March 2027 without a substantive hearing, effectively constituting a final interdict without a final hearing. A Scale C costs order is threatened against a destitute litigant, which is prima facie inconsistent with the Biowatch principle. The Deneys Reitz letter of 4 June 2026 threatened the complainant with contempt of court for petitioning Parliament. The SAHRC acknowledged the seriousness of the situation and went silent. SAPS registered a criminal case and then conducted no investigation.

PARLIAMENTARY RECOMMENDATION

Parliament must request the Judge Presidents of the Western Cape High Court and the Labour Court to investigate the conduct of the respective Registrars and the procedural failures identified in this report. Parliament must refer the conduct of Deneys Reitz Inc. to the Legal Practice Council. Parliament must request IPID to investigate the conduct of the Delft Police Station officer. Parliament must summon the SAHRC Chairperson to explain the non-investigation of a complaint that explicitly invoked Sections 10, 26, 27, and 33 of the Constitution.

Executive Summary – Standing Committee on Public Accounts (SCOPA)

The Government Employees Medical Scheme (GEMS) manages the medical benefits of 1.9 million public servants. It is funded by member contributions – public sector payroll deductions – that constitute, in their aggregate, public funds. GEMS contracts Medscheme Holdings to administer its benefits. When the administrator's own employee identified a systemic clinical risk in the data platform used to generate clinical compliance letters to beneficiaries, and formally disclosed that risk to GEMS on two occasions (2 April 2026 and 13 April 2026), GEMS did not respond substantively. It did not investigate. It did not commission an independent review. It did not inform its beneficiaries. The actuarial implications of the Gexus-DMS integration gap – false non-adherence data inflating risk loading calculations – remain unquantified and unaddressed. AfroCentric's Integrated Annual Report 2025, published while the company held the GEMS government contract, omitted material information that any investor, government counterparty, or oversight body would consider relevant: a Section 162 director delinquency application, a B-BBEE Commission investigation with a potential R580 million fine, and an active criminal case.

PARLIAMENTARY RECOMMENDATION

SCOPA must request the Auditor-General to commission a forensic audit of Medscheme's administration of GEMS, with specific focus on the Gexus-DMS integration gap, its actuarial implications, and whether member contributions were properly managed during the affected period. SCOPA must request GEMS to table a report on its oversight of Medscheme and on the steps taken to address the identified clinical risk. SCOPA must request the relevant portfolio committee to require AfroCentric to restate its Integrated Annual Report 2025.

Executive Summary – Portfolio Committee on Police

On 23 February 2026, the complainant attended Delft Police Station to open a criminal case under the Unemployment Insurance Contributions Act 4 of 2002 for non-remittance of UIF contributions deducted from his salary in June 2025, and under the common law for false written compliance representations by two named Medscheme HR officials. The evidence was documentary, primary, and unambiguous. A police officer at Delft Police Station refused to open the case. The officer falsely represented to the complainant that he was the station commander – a misrepresentation that the complainant recorded on audio. After escalation to Brigadier Naidoo, the case was registered as CAS 705/3/2026 and transferred to Woodstock Police Station. As at June 2026, more than three months after registration, no detective has been assigned, no statement has been taken from the complainant, and no investigation has been conducted. The internal complaint against the Delft officer was closed on the basis that a case number had been allocated – without addressing the false representation.

PARLIAMENTARY RECOMMENDATION

Parliament must summon the SAPS National Commissioner to account for the three-month stagnation of CAS 705/3/2026 and for the non-investigation of documented statutory offences. Parliament must request IPID to investigate the Delft officer for falsely claiming to be station commander and for refusing

to open a criminal case. Parliament must request a written explanation from the SAPS for the closure of the internal complaint without addressing the false representation.

PART EIGHT: THE WHISTLEBLOWER AND THE SYSTEMIC CLINICAL RISK

This section provides a consolidated profile of the whistleblower and the systemic clinical risk that his disclosure set in motion. It is designed as a reference section for committee members who wish to understand the foundational facts before engaging with the institutional analyses in Parts Nine through Twenty.

FINDING	EVIDENCE / SOURCE
Whistleblower	Sr. Wilbur William Matthee. Registered Nurse Specialist (SANC No. 15831886). BTech General Nursing. Postgraduate Diploma in Health Services Management (NQF Level 8, CPUT, awarded with distinction). Founder, EthicHawks Forensic Governance & Compliance Consultancy.
Employer	Medscheme Holdings (Pty) Ltd, a wholly owned subsidiary of AfroCentric Investment Corporation Limited (JSE: ACT). Medscheme is the accredited administrator for GEMS.
Principal Scheme	Government Employees Medical Scheme (GEMS) – 1.9 million members including South African public servants and their dependants.
Role at Medscheme	Specialist: Care Manager (as recruited); contracted as Care Coordinator (Grade B4M1). Active Disease Risk Management Unit. Responsible for clinical case management of GEMS beneficiaries with chronic and complex conditions, including management of data from the Gexus and DMS platforms.
Risk Identified	Gexus-DMS integration gap: a structural miscommunication between the Gexus chronic disease management platform and the DMS document management system that caused false non-adherence letters to be generated for GEMS beneficiaries who were in fact adherent to their medication regimes. Confirmed by Medscheme's own internal Root Cause Analysis (RCA). The gap affected at minimum GEMS Case 000655789 (a 75-year-old widow collecting cholesterol medication monthly) and potentially all beneficiaries managed through both platforms.
Protected Disclosure	23 October 2025. Formal protected disclosure under PDA Section 1: the complainant identified the data error, refused to use false data to interrogate an elderly patient, and made a formal written disclosure to his manager and the GRC system. Followed by Priority 1 Material Risk Report to the Board of AfroCentric on 9 December 2025 (38 attachments).
Retaliation (Summary)	Unlawful salary deduction (40.47%, July 2025); gender-identity harassment ("Wilma", September 2025); retaliatory insubordination charge; recorded admission of retaliatory intent by manager (31 October 2025); suppression of Board escalation by CRO (18 December 2025); constructive dismissal (12 December 2025); false UI-19 (Code 6 instead

FINDING	EVIDENCE / SOURCE
	of Code 7); suppression of exit documents including Certificate of Service.
Medical Consequences	Clinically diagnosed severe depression and Post-Traumatic Stress Disorder (Dr Jacques Malan, FCPSYCH(SA)), formally attributed to occupational causation. Admitted to Akeso Stepping Stones Clinic (18 March 2025). Psychiatric letter provided to employer 16 May 2025 – employer placed on formal notice of disability before the primary disclosure events.
Current Material Circumstances	Homeless since 2 April 2026. No running water since January 2026. Personal possessions seized by loan sharks. Deregistered by SANC (annual fee of R870 unpaid – direct consequence of unlawful deduction). Unable to enrol in CPUT Postgraduate Diploma in Occupational Health Nursing (2026 intake) due to financial incapacity. Self-represented in nine concurrent proceedings.
Gag Order	Western Cape High Court interim interdict (Case No. 2026-086906), granted ex parte on 16 April 2026 after the employer's attorneys concealed the complainant's answering affidavit. Restrains the complainant from disclosing the systemic clinical risk to trade unions and regulators. Rule nisi extended to 1 March 2027. Currently challenged in a contempt defence affidavit and in this parliamentary petition.
Concurrent Proceedings	Nine concurrent proceedings: High Court Case 2026-086906 (gag order); High Court Case 2026-066952 (Section 162 application, rescission pending); Labour Court Case 2026-083299 (constructive dismissal); Labour Court Case 2026-077546 (BCEA salary deduction claim); B-BBEE Commission Case 5/02/2026; SAPS CAS 705/3/2026; JSE Ref JvS/166792; FSCA Ref 1-370127; Formal complaint to SAHRC.

PART NINE: THE FOUR HEALTH REGULATORS – A CARTEL OF SILENCE

The four statutory health regulators – the Council for Medical Schemes (CMS), the Health Professions Council of South Africa (HPCSA), the South African Nursing Council (SANC), and the South African Pharmacy Council (SAPC) – were each approached by the complainant with a specific complaint directly within their statutory mandates. Not one of them provided a substantive response. Together, they constitute what this report describes as a cartel of silence: a pattern of coordinated non-response by the very institutions that are mandated to protect patients, professionals, and the public interest in healthcare.

This is not a coincidence of administrative failure. It is a structural failure of regulatory architecture. And it is directly relevant to the NHI: every one of these four bodies will be tasked with oversight of NHI-accredited providers. If they cannot investigate a documented Priority 1 clinical risk in the existing system, they will not investigate fraud, fronting, or clinical failures in the NHI.

9.1 Council for Medical Schemes (CMS)

The CMS is established under Section 7 of the Medical Schemes Act 131 of 1998 with a mandate to protect the interests of beneficiaries of medical schemes, investigate complaints against schemes and their administrators, and monitor and enforce compliance with the MSA. The Gexus-DMS integration gap is precisely the kind of administrator maladministration that the CMS was designed to investigate. The Root Cause Analysis generated by Medscheme’s own systems confirmed the structural design flaw. The impact on a 75-year-old GEMS beneficiary – receiving false non-adherence letters despite collecting her medication monthly – is documented in the complainant’s disclosure and in the daughter’s letter to Medscheme.

FINDING	EVIDENCE / SOURCE
Declined jurisdiction (first time, 4 February 2026)	CMS email of 4 February 2026 refers the complainant to the HPCSA. The CMS does not explain which provision of the MSA limits its jurisdiction. No jurisdictional analysis is provided. The referral to the HPCSA is circular – the HPCSA subsequently refers to SANC.
Declined jurisdiction (second time, 7 May 2026)	CMS email of 7 May 2026 demands a new complaint form from the complainant, despite the High Court Interim Order having been granted on 16 April 2026. Compliance with the demand would have required the complainant to disclose confidential information covered by the interim interdict – a demand that effectively constituted a contempt trap.
Disparate treatment – Discovery Health (January 2026)	In January 2026, the CMS exercised jurisdiction over a system error at Discovery Health that generated incorrect chronic medication correspondence to beneficiaries. This is materially the same type of error as the Gexus-DMS integration gap at Medscheme/GEMS. The CMS investigated Discovery Health but declined Medscheme/GEMS. No explanation for the differential treatment has been provided. This is the strongest single indicator of regulatory capture in this case.
Core ethical question unanswered (14 April 2026)	The complainant posed the following question to the CMS, HPCSA, SANC, and SAPC jointly on 14 April 2026: “May a registered health professional refuse an employer’s instruction to act on data known to be clinically false when

FINDING	EVIDENCE / SOURCE
	that action could harm a patient?" The CMS did not answer. This question is foundational to the governance of every clinical platform that will operate under the NHI.

CONCLUSION: The CMS acted to protect Medscheme/GEMS, not its 1.9 million beneficiaries. It is the regulator mandated to oversee NHI accreditation. Its documented pattern of selective enforcement – accountability for Discovery Health, protection for Medscheme – is the clearest evidence of regulatory capture in this report.

KEY QUESTIONS FOR THE COMMITTEE

- Why did the CMS decline jurisdiction twice despite Sections 7, 44, 47, and 58 of the MSA conferring a clear mandate to investigate administrator maladministration?
- On what legal basis did the CMS exercise jurisdiction over Discovery Health's similar error in January 2026 while refusing to investigate Medscheme's Gexus-DMS integration gap?
- Why did the CMS demand a new complaint form from the complainant after the High Court Interim Order was granted, when compliance with that demand would have risked contempt of court?
- What investigation has the CMS conducted into the clinical data quality of Medscheme's GEMS administration since being formally notified of the integration gap?

9.2 Health Professions Council of South Africa (HPCSA)

The HPCSA is the statutory body established to set professional standards and investigate misconduct by registered health professionals. The complainant's complaint raised a foundational question of professional ethics: whether a registered health professional may be compelled by an employer to act on data known to be clinically false, in circumstances where that action could harm a patient. This question is not abstract. It is the exact situation the complainant faced on 23 October 2025. The HPCSA's answer to this question – or rather, its silence – tells every health professional in South Africa what to expect when they face the same dilemma.

FINDING	EVIDENCE / SOURCE
No response to urgent joint ethical ruling request (14 April 2026)	The complainant wrote to the CMS, HPCSA, SANC, and SAPC jointly on 14 April 2026 requesting an urgent ethical ruling. The HPCSA did not respond. The imminent clinical risk and the urgent nature of the request were clearly stated.
Jurisdictional ping-pong	On 11 February 2026, the HPCSA referred the complainant to the SANC, which subsequently referred him back. This circle of referrals is itself a form of institutional obstruction – each body declining to act and directing the complainant to another body that will also decline.
Silence on Priority 1 clinical risk	Despite being formally notified of a Priority 1 clinical risk affecting 1.9 million GEMS beneficiaries, the HPCSA did not trigger any investigation under the Health Professions Act, did not convene the relevant Professional Conduct Committee, and did not issue any guidance to registered health professionals about their duties in similar situations.

FINDING	EVIDENCE / SOURCE
CONCLUSION: The HPCSA's silence is a constructive refusal to uphold its statutory mandate. By failing to answer the core ethical question, the HPCSA has left every health professional in South Africa without guidance on the most fundamental ethical dilemma they may face: the duty to the patient vs the instruction of the employer.	

9.3 South African Nursing Council (SANC)

The SANC is the statutory body established under the Nursing Act to regulate nursing education, practice, and professional conduct. The complainant filed a formal professional misconduct complaint on 10 February 2026 against two registered nurses employed by Medscheme: the nurse who issued the instruction to contact the 75-year-old patient on false data, and the nurse who swore the CCMA affidavit describing the complainant's clinically diagnosed PTSD as a "cynical excuse." Both complaints are within the SANC's direct mandate. Neither has been acknowledged.

FINDING	EVIDENCE / SOURCE
Formal complaint not acknowledged (SANC/MEDSCHEME/MATTHEE/20260105)	Filed 10 February 2026. As at June 2026 – more than four months later – the complaint has not been acknowledged, assigned a case officer, or processed in any identifiable way.
Silence for 60+ days on Priority 1 risk	Remained silent in the face of a formally notified Priority 1 systemic risk affecting 1.9 million GEMS beneficiaries. SANC's silence means the clinical risk remains addressed only by the complainant's protected disclosure – which is itself suppressed by an interim interdict.
Deregistered the complainant	The complainant was deregistered by SANC for non-payment of R870 in annual registration fees. The non-payment is a direct consequence of the employer's unlawful deduction of R14,811.77, which left the complainant without the means to pay. SANC deregistered the very person who identified the clinical risk and whose deregistration was itself caused by the employer's unlawful conduct. This is institutional injustice.
Jurisdictional ping-pong	SANC participated in circular referrals between CMS, HPCSA, and SAPC, with no body willing to accept responsibility for answering the core ethical question.
CONCLUSION: SANC's inaction directly protected Medscheme/GEMS and enabled the ongoing patient safety risk. SANC then compounded the harm by deregistering the very whistleblower it failed to protect. The institutional chain – from failure to protect, to failure to investigate, to active harm through deregistration – is documented in full.	

9.4 South African Pharmacy Council (SAPC)

The SAPC is the statutory body established under the Pharmacy Act to regulate pharmacy practice and investigate the professional conduct of registered pharmacists. Medscheme's Responsible Pharmacist, Lisa Mari, oversaw the clinical pharmacological aspects of the disease risk management process that relied on Gexus-DMS data, including the non-adherence letters. The SAPC received a formal complaint about Lisa Mari's professional conduct in the context of the clinical data error. The SAPC declined jurisdiction.

FINDING	EVIDENCE / SOURCE
Jurisdiction declined over formal complaint	The SAPC declined jurisdiction over the formal complaint against Lisa Mari, Responsible Pharmacist, despite Section 49 of the Pharmacy Act conferring a clear mandate to investigate professional misconduct by registered pharmacists. No written jurisdictional analysis was provided.
Hesitation to rule on core ethical question	During the SAPC's Professional Conduct Committee meeting of 14 April 2026, committee members expressed hesitation to rule on the core ethical question (whether a registered professional may refuse an instruction based on known false clinical data). No ruling was issued. No written decision was provided. The committee went silent after the meeting.
CONCLUSION: SAPC's declination and hesitation complete the four-regulator failure pattern. CMS, HPCSA, SANC, and SAPC – the four statutory bodies that together regulate virtually every healthcare professional who will be involved in NHI implementation – collectively failed to answer a single foundational ethical question. This is a cartel of silence.	

PART TEN: THE PRINCIPAL SCHEME – GEMS: COMPLICITY THROUGH SILENCE

The Government Employees Medical Scheme (GEMS) is not a passive bystander in this report. It is the principal scheme – the entity that contracted Medscheme Holdings to administer medical benefits for 1.9 million public servants. GEMS's governance obligations include oversight of its administrator's compliance with accreditation standards, risk escalation protocols, and member protection requirements. When a member of Medscheme's own clinical staff formally disclosed a systemic clinical risk affecting GEMS beneficiaries directly to GEMS, the scheme had both the authority and the obligation to act. It did neither.

FINDING	EVIDENCE / SOURCE
Protected disclosure of 2 April 2026 – no substantive response	The complainant formally disclosed the Gexus-DMS integration gap to GEMS on 2 April 2026, identifying the risk to beneficiaries' clinical data integrity and the potential for false non-adherence flags in chronic disease management records. GEMS did not acknowledge the substance of the disclosure, did not commission an independent review, and did not instruct Medscheme to investigate.
Protected disclosure of 13 April 2026 – no substantive response	A second, more detailed disclosure was sent to GEMS on 13 April 2026, following the grant of the interim interdict two days earlier. GEMS again did not respond substantively. The silence of the principal scheme at this critical moment is itself evidence of institutional complicity.
Failure to oversee administrator compliance	GEMS's accreditation framework and administration agreement with Medscheme include provisions for risk escalation, clinical quality assurance, and member protection. These provisions were not enforced when a formal risk disclosure was received from within the administrator's own clinical team.
Actuarial implications unaddressed	The Gexus-DMS integration gap generated false non-adherence data. False non-adherence data inflates risk loading in actuarial models. Inflated risk loading affects premium calculations. Premium calculations directly determine the level of contributions deducted from the salaries of 1.9 million public servants. The actuarial implications of this gap have not been independently reviewed, disclosed, or remedied.

CONCLUSION: GEMS's silence constitutes institutional complicity in the retaliation against a protected whistleblower and in the ongoing patient safety risk to its 1.9 million members. A government medical scheme that cannot oversee its administrator's compliance with clinical data standards is not fit to participate in the NHI.

KEY QUESTIONS FOR THE COMMITTEE

- Why did GEMS not commission an independent clinical data audit following the protected disclosures of 2 and 13 April 2026?
- What oversight mechanisms does GEMS have over Medscheme's clinical data quality, and why did those mechanisms not trigger an investigation when the Gexus-DMS gap was disclosed?

FINDING	EVIDENCE / SOURCE
• Has the actuarial impact of false non-adherence data in the Gexus-DMS integration gap been independently quantified? • Will GEMS now commission an independent investigation and report to the Portfolio Committee on Health?	

PART ELEVEN: DEPARTMENT OF EMPLOYMENT AND LABOUR – ADMINISTRATIVE ABANDONMENT

The Department of Employment and Labour (DEL) has three specific statutory obligations directly engaged by the facts of this case: enforcement of the BCEA Section 34(2) deduction cap; enforcement of UIF contribution remittance under the Unemployment Insurance Contributions Act 4 of 2002; and correction of false UI-19 declarations under the Unemployment Insurance Act 63 of 2001. The DEL failed all three obligations. This failure is not a matter of discretion. These are non-discretionary statutory duties.

FINDING	EVIDENCE / SOURCE
BCEA Section 34(2) deduction cap not enforced	The employer deducted 40.47% of the complainant's salary in a single pay cycle, exceeding the 25% cap by 15.47 percentage points. DEL was formally notified. DEL did not issue a Compliance Order under BCEA Section 69. The employer's own written statement – "our policy doesn't permit for staggering" – constitutes an admission of non-compliance with the cap that DEL could have acted on immediately.
UIF contributions not enforced (June 2025)	The uFiling portal shows "No Information" for the complainant's UIF contributions for June 2025, the month of the unlawful deduction. DEL was formally notified. DEL refused to compel remittance, claiming the CCMA has exclusive jurisdiction. This is an error of law: the CCMA does not have jurisdiction over UIF contribution remittance. The UIF Act gives DEL primary enforcement authority.
False UI-19 not corrected	The employer filed a UI-19 declaring Code 6 (Resigned). The complainant's resignation letter explicitly invokes LRA Section 186(1)(e) – constructive dismissal – which should be Code 7. DEL Principal Inspector Kutuka closed the investigation file on 15 April 2026 with the jurisdictional error, addressing his response to a generic "Mr/Mrs/Ms." DEL's failure to correct the UI-19 continues to block the complainant's access to UIF benefits, compounding the homelessness directly caused by the employer's unlawful deduction.
60+ days of silence on multiple escalations	The complainant issued multiple formal escalations to DEL between January and May 2026. DEL remained silent on most of them, responding substantively only to close the file with an error of law.
CONCLUSION: DEL's conduct violates Section 27(1)(c) of the Constitution (right to social security) and Section 33 (just administrative action). A department that closes a UIF investigation file with a legal error, addresses its response to a generic "Mr/Mrs/Ms," and leaves a homeless whistleblower without access to UIF benefits has failed its most basic constitutional mandate.	
KEY QUESTIONS FOR THE COMMITTEE	
- Why did DEL not issue a Compliance Order under BCEA Section 69 when the employer's own admission confirmed the 25% cap was not applied? - On what legal authority did DEL claim the CCMA has exclusive jurisdiction over UI-19 fraud and UIF contribution non-remittance?	

FINDING	EVIDENCE / SOURCE
• Will the Director-General personally issue a formal Compliance Order requiring Medscheme to remit the outstanding UIF contributions and correct the UI-19 to Code 7 within 30 days?	

PART TWELVE: THE B-BBEE COMMISSION – DECLINATION OF FRONTING COMPLAINT

The B-BBEE Commission’s mandate is to investigate complaints of fronting – the practice by which a Black person or entity is used to achieve B-BBEE scorecard compliance without that person receiving the economic benefits, management authority, or professional recognition associated with the position. Title suppression – recruiting a Black professional for a senior role and contracting him at a junior grade while using his senior credentials for B-BBEE score purposes – is fronting under Section 1(c) of the B-BBEE Act.

The complainant’s fronting complaint is supported by three categories of primary source evidence that are, individually and collectively, sufficient to warrant investigation: interview invitations clearly describing a “Specialist: Care Manager” role (January 2024); an employment contract classifying the same role as “Care Coordinator” Grade B4M1 (9 April 2024); and a written admission by Senior HR Business Partner Cheryl-Dawn Modern that the naming convention changes were “not applied to your department” (19 January 2026). The last of these is, in the complainant’s submission and in this report’s forensic assessment, the single most important document in the fronting complaint. It is a direct written admission of title suppression by the employer’s own HR representative.

FINDING	EVIDENCE / SOURCE
Title suppression confirmed by primary source evidence	Three categories of employer-generated documents confirm the role was advertised as Specialist: Care Manager and contracted as Care Coordinator (Grade B4M1). The salary differential between the two grades is material.
Written admission of naming convention suppression	Cheryl-Dawn Modern’s email of 19 January 2026: “The naming convention changes were not applied to your department.” This is not an inference or allegation. It is the employer’s own written admission that the professional naming convention – which would have reflected the complainant’s actual seniority – was withheld from his employment record.
Commission declined to investigate (Notice of Non-Investigation, 30 March 2026)	The Commission issued a Notice of Non-Investigation classifying the complaint as a labour dispute. This is an error of law. A labour dispute concerns the terms and conditions of employment between employer and employee. A B-BBEE fronting complaint concerns the exploitation of a Black professional’s credentials for scorecard purposes in a company report that is used to obtain or retain government contracts and to achieve compliance with transformation legislation. These are different legal matters with different regulatory consequences.
Section 13K subpoena powers refused	The complainant specifically requested the Commission to use its Section 13K subpoena powers to obtain the Teams recording of the naming convention discussion, in which the suppression decision was allegedly made. The Commission refused. A regulatory body with statutory subpoena powers that refuses to use them in the face of written evidence of suppression has made a deliberate choice not to investigate.
Payroll anomaly (potential statutory misrepresentation)	A forensic payroll audit identified a discrepancy of R5,948.62 between the complainant’s declared UIF

FINDING	EVIDENCE / SOURCE
	figures and his payslip Normal Earnings of R36,588.81. This potential statutory misrepresentation in UIF filings is a separate evidentiary category that the Commission also declined to investigate.
CONCLUSION: The B-BBEE Commission's declination of Case 5/02/2026 is a dereliction of statutory duty and constitutes regulatory capture. A company that holds the GEMS administration government contract – and that reports a Level 1 B-BBEE Management Control score of 14.67/19 – used a Black NQF Level 8 professional's credentials for scorecard purposes while suppressing his title and refusing to pay the salary appropriate to his role. The Commission refused to investigate despite primary source written evidence.	
KEY QUESTIONS FOR THE COMMITTEE	
• Why did the Commission classify title suppression supported by a written admission from the employer's HR Business Partner as a labour dispute rather than as a fronting practice under B-BBEE Act Section 1(c)? • Why were Section 13K subpoena powers not deployed to obtain the Teams recording of the naming convention discussion? • Will the Commission reopen Case 5/02/2026, investigate it on its merits as a B-BBEE fronting complaint, and deploy its full statutory powers including Section 13K? • What contractual consequences does a finding of B-BBEE fronting have for a company that holds a government administration contract under the GEMS scheme?	

PART THIRTEEN: SOUTH AFRICAN HUMAN RIGHTS COMMISSION (SAHRC) – CHAPTER 9 ABDICATION

The South African Human Rights Commission is a Chapter 9 institution established by Section 181(1)(b) of the Constitution. Its mandate, under Section 184, is to promote respect for human rights and a culture of human rights, to promote the protection, development, and attainment of human rights, and to monitor and assess the observance of human rights in South Africa. It has broad investigative powers, including the power to investigate on its own initiative under Section 14 of the SAHRC Act. It has the power to issue subpoenas, demand documents, and refer matters to the NPA.

The complainant's complaint to the SAHRC invoked four specific constitutional rights: Section 10 (dignity – the “Wilma” harassment and the sworn characterisation of his PTSD as a “cynical excuse”); Section 26 (housing – the homelessness resulting from the unlawful deduction and regulatory failure); Section 27 (food, water, healthcare – the loss of running water and healthcare access since January 2026); and Section 33 (just administrative action – the conduct of DEL, CMS, SANC, SAPS, and the CCMA). These are not peripheral complaints. They are core constitutional rights violations, documented by primary source evidence, affecting a South African citizen in real time.

FINDING	EVIDENCE / SOURCE
Acknowledged seriousness; then went silent (10 February 2026)	The SAHRC's response of 10 February 2026 acknowledged “the seriousness of the concerns” raised by the complainant. This is not a rejection. It is an acknowledgment of merit. The SAHRC then redirected the complainant to the CCMA and the Public Protector – neither of which has jurisdiction over the constitutional rights violations identified (the CCMA has no mandate over housing, water, or dignity violations by a private employer; the Public Protector investigates state organs, not private companies like Medscheme).
13 March 2026 update – no response	On 13 March 2026, the complainant submitted an eight-page update to the SAHRC documenting: a forensic payroll anomaly (R5,948.62 discrepancy in UIF declarations); the CCMA's institutional bias; the Labour Court portal obstruction; SAPS's refusal to open a criminal case; and confirmed imminent homelessness. The SAHRC did not acknowledge or respond to this update.
Own-initiative investigation power not triggered	By March 2026, the complainant had documented 11 institutional failures in a single case. The SAHRC's own Section 14 power enables it to initiate investigations “on its own initiative.” This is precisely the type of case that warrants an own-initiative investigation: a cascade of regulatory and judicial failure affecting the constitutional rights of a single citizen in real time.
Redirect to institutions without jurisdiction	Referring the complainant to the CCMA for housing and water violations, and to the Public Protector for private employer misconduct, is not a substantive response. It is a recycling of the complainant into an institutional maze – the same maze from which he was trying to escape when he approached the SAHRC.
CONCLUSION: A Chapter 9 institution that acknowledges the seriousness of constitutional rights violations and then goes silent is not protecting human rights. It is filing them away. The	

FINDING	EVIDENCE / SOURCE
SAHRC’s conduct in this case – acknowledgment without investigation, redirection without jurisdiction analysis, silence after confirmed destitution – represents a constitutional dereliction of the highest order.	
KEY QUESTIONS FOR THE COMMITTEE	
• Why did the SAHRC not open a formal investigation after acknowledging the seriousness of the documented constitutional rights violations? • Why was the 13 March 2026 update – confirming imminent homelessness and documenting eleven institutional failures – not acknowledged or acted upon? • Why did the SAHRC not trigger its own-initiative investigation power under SAHRC Act Section 14? • Will the SAHRC now open an own-initiative investigation into the systemic pattern of regulatory failure documented in this report and its constitutional implications?	

PART FOURTEEN: THE SOUTH AFRICAN POLICE SERVICE (SAPS) – REFUSAL TO INVESTIGATE CRIMINAL OFFENCES

The South African Police Service is constitutionally mandated under Section 205(3) of the Constitution to prevent, combat, investigate, and solve crime. UIF non-remittance is a criminal offence under Section 56 of the Unemployment Insurance Contributions Act 4 of 2002. Making a false statement in a written declaration to a statutory body – including a false UI-19 – is a criminal offence. Making false written compliance representations to induce reliance is fraud at common law. The complainant attended Delft Police Station on 23 February 2026 to open a criminal case for all three categories of offence. A police officer refused to open the case and falsely claimed to be the station commander. The interaction was audio-recorded.

FINDING	EVIDENCE / SOURCE
Refusal to open case at Delft Police Station (23 February 2026)	A police officer at Delft Police Station, whose actual rank and identity are known to the complainant, refused to open a criminal case for UIF non-remittance and false compliance representations. The officer stated that this was a labour matter, not a criminal matter – which is incorrect as a matter of law. The Unemployment Insurance Contributions Act 4 of 2002 expressly creates criminal liability for non-remittance.
Officer falsely claimed to be station commander	The officer falsely represented to the complainant that he was the station commander of Delft Police Station. This misrepresentation was intended to confer authority on the refusal and to deter the complainant from escalating. The interaction was audio-recorded by the complainant. The recording is available to IPID and to Parliament.
CAS 705/3/2026 registered but stagnant	After escalation to Brigadier Naidoo, the case was registered as CAS 705/3/2026 and transferred to SAPS Woodstock. As at June 2026, more than three months after registration, no detective has been assigned, no statement has been requested from the complainant, and no investigative steps have been taken. This is not delay. It is non-investigation with a case number.
Internal complaint closed improperly	The internal complaint about the Delft officer was closed on the basis that a case number had been allocated – without addressing the false representation about the officer's rank or the original refusal to open the case. The allocation of a case number to a case that is not being investigated does not constitute the discharge of the SAPS's Section 205(3) constitutional mandate.
CONCLUSION: The SAPS failed its constitutional duty to investigate documented criminal offences. The registration of CAS 705/3/2026 without investigative follow-up is a procedural formality that creates the appearance of compliance while providing none of the substance. The audio-recorded false representation by the Delft officer is a criminal matter in its own right and warrants referral to IPID.	

PART FIFTEEN: THE COMMISSION FOR CONCILIATION, MEDIATION AND ARBITRATION (CCMA)

Case References: WECT24486-25 (Constructive Dismissal) and WECT23564-25 (Condonation). Conciliation: 9 January 2026. Commissioner: Ntabiseng Ngwane. Provincial Senior Commissioner: Vusumzi Landu. Employer representative: Vijay Makenjee (Deneys Reitz Inc.) and Xolile Manyati (AfroCentric HR).

The CCMA conciliation of 9 January 2026 is the centrepiece of the institutional bias documented in this report. It was not a conciliation. It was a reenactment of the employer's power. The Commissioner arrived with a closed mind, applied no law, provided no assistance, tolerated contempt, and then participated in – or at least permitted – a cover-up by the Provincial Senior Commissioner.

FINDING	EVIDENCE / SOURCE
Prejudgment before evidence	Commissioner Ngwane stated “nothing here relates to a protected disclosure” within the opening minutes of the conciliation, before the complainant had presented a single document, called a single fact, or cited a single provision of the PDA. This is prejudgment – a decision made before hearing the case. It is the antithesis of adjudication.
“Google it” – failure to assist destitute litigant	When the homeless, mentally distressed, self-represented complainant asked the Commissioner how to find the Labour Court address, the Commissioner responded: “You’re gonna have to... Google it or something like that.” This violates LRA Section 138(1) (the Commissioner’s duty to conduct the proceedings) and the CCMA’s own helping-hand principle, which requires Commissioners to assist self-represented litigants in understanding the process. It also reflects the absence of basic human decency in an institution that is supposed to be accessible to ordinary workers.
BCEA Section 34 analysis entirely absent	The Commissioner never asked for or reviewed the payslip. The Commissioner never cited the 25% deduction cap. The Commissioner never asked the employer to produce written consent for the deduction. The Commissioner uncritically accepted the employer’s characterisation of the deduction as “no work, no pay,” despite the employer’s own written admission that “our policy doesn’t permit for staggering” – a direct confirmation that the 25% cap was not applied.
Systemic bias against self-represented complainant	During the conciliation, the Commissioner challenged none of the employer’s statements, allowed the employer’s representative to dominate the proceedings, did not intervene when the representative laughed at the complainant’s settlement demand, and did not provide any procedural assistance to the self-represented complainant.
CCMA mailbox full – systemic barrier to filing	The complainant’s replying affidavit and amendment request were rejected by the CCMA’s own filing system with a “mailbox full” error. This is not the complainant’s failure. It is an institutional failure – a statutory dispute resolution body whose email system cannot receive the filings it is mandated to process.

FINDING	EVIDENCE / SOURCE
PSC cover-up by Vusumzi Landu	The Provincial Senior Commissioner's formal response (28 January 2026) addressed only a fraction of the allegations raised in the complainant's complaint, defended the indefensible (characterising the Commissioner's "Google it" response as acceptable), and victim-blamed the complainant for recording the conciliation session – a recording that the complainant was entitled to make under the relevant rules.
Potential perjury by Xolile Manyati	The employer's CCMA opposing affidavit, sworn by Xolile Manyati on 10 December 2025, characterised the complainant's clinically diagnosed severe depression and PTSD as a "cynical excuse". This affidavit was filed one day after the Priority 1 Material Risk Report was escalated to the Board – suggesting that the timing was retaliatory. The characterisation of a formally diagnosed, clinically documented psychiatric condition as a "cynical excuse", in a sworn court document, is a potential perjury offence.
CONCLUSION: The CCMA conciliation of 9 January 2026 was not a neutral forum for dispute resolution. It was an institutional endorsement of employer power against a destitute, mentally distressed, self-represented whistleblower. The internal complaint process is a cover-up mechanism, not an accountability mechanism. Parliament must establish an independent external complaints body for CCMA commissioner misconduct.	

PART SIXTEEN: THE WHISTLEBLOWER HOUSE (TWBH) AND SCHROTER ATTORNEYS – FLAWED ASSESSMENT AND ABANDONMENT

The Whistleblower House (TWBH) exists specifically to provide legal support and advocacy to whistleblowers in South Africa. It is the institution most directly mandated to assist a person in exactly the complainant's situation. The complainant approached TWBH and was assessed by attorney Marco van der Walt of Schroter Attorneys. Van der Walt's assessment was legally incorrect in three material respects, and TWBH's rejection – based entirely on van der Walt's flawed opinion – abandoned a destitute, mentally distressed, homeless nurse to self-represent in nine concurrent proceedings.

FINDING	EVIDENCE / SOURCE
Legal error 1: "No work no pay" mischaracterisation	Van der Walt characterised the unlawful 40.47% deduction as "no work no pay". This is legally incorrect. The "no work no pay" principle in South African labour law applies to the non-payment of wages during periods of protected industrial action. It does not authorise a deduction that exceeds the 25% cap in BCEA Section 34(2). The employer's own written admission that "our policy doesn't permit for staggering" directly contradicts the "no work no pay" characterisation and confirms non-compliance with the cap.
Legal error 2: "Isolated system glitch" characterisation	Van der Walt dismissed the systemic Gexus-DMS integration failure as "a single isolated incident of a system glitch." The Root Cause Analysis, generated by Medscheme's own systems, confirmed a structural design flaw affecting the integration between two platforms. A structural design flaw confirmed by a Root Cause Analysis is not a glitch. This characterisation reflects either a failure to read the RCA or a deliberate minimisation of its significance.
Legal error 3: "Bad faith" allegations about regulatory disclosures	Van der Walt suggested that the complainant's disclosures to regulators might constitute "bad faith." This is legally incorrect and deeply harmful. Protected disclosures made in good faith to appropriate regulatory authorities are expressly protected under Section 8 of the PDA. Suggesting that regulatory disclosures might be in bad faith actively discourages the exercise of a constitutional and statutory right.
Security alert ignored	The complainant reported to van der Walt that emails were vanishing from his inbox – a potential security breach with significant evidential implications in a case involving nine concurrent proceedings. Van der Walt did not engage with this report.
TWBH rubber-stamped rejection	TWBH rejected the complainant's application without commissioning an independent review of van der Walt's assessment. A whistleblower protection organisation that relies entirely on a single attorney's potentially flawed opinion to reject the most vulnerable category of whistleblower it was created to serve has failed its foundational mandate.

FINDING	EVIDENCE / SOURCE
CONCLUSION: TWBH and Schroter Attorneys failed the very person their mandate was designed to protect. Their errors did not merely leave the complainant without assistance. They actively reinforced the institutional narrative that his disclosure was not a protected disclosure and that his regulatory engagements might be in bad faith. The conduct of both institutions should be referred to the Legal Practice Council and the Public Protector.	

PART SEVENTEEN: THE JUDICIARY – JUDICIAL OBSTRUCTION AND ABUSE OF PROCESS

Cases: 2026-086906 (Western Cape High Court – gag order); 2026-066952 (Western Cape High Court – Section 162 application, struck); 2026-083299 (Labour Court – constructive dismissal); 2026-077546 (Labour Court – BCEA salary deduction).

The judiciary represents the last institutional resort for a person whose rights have been violated and whose regulatory remedies have been exhausted. This report documents not merely a failure of judicial process, but active judicial obstruction through administrative conduct, concealment, and the granting of a gag order that suppresses a protected disclosure. The failures documented in this section are, in some respects, the most serious in the report – because the courts are the ultimate constitutional backstop, and when they fail, there is nowhere left to go.

17.1 High Court – The Gag Order and Fraud on the Court

On 16 April 2026, the Western Cape High Court granted an ex parte interim interdict in Case No. 2026-086906. The interdict restrains the complainant from disclosing the systemic clinical risk to trade unions and regulators. It overrides Section 9 of the Protected Disclosures Act, which expressly protects disclosures to regulatory authorities. The rule nisi has been extended to 1 March 2027. The entire edifice of this interdict rests on a fraud.

FINDING	EVIDENCE / SOURCE
Answering affidavit concealed from the court	The complainant had filed an answering affidavit in opposition to the employer's interdict application. The employer's attorneys (Deneys Reitz Inc.) did not hand up this affidavit at the ex parte hearing of 16 April 2026 and did not inform the court that the matter was opposed. This is a breach of the fundamental ex parte duty of full and frank disclosure. It is not a procedural oversight. It is a deliberate concealment of opposition that induced the court to grant relief it would not have granted had it known the matter was contested.
Interdict overrides the PDA	Section 9 of the PDA protects disclosures made by a whistleblower to regulatory bodies, trade unions, and law enforcement agencies. The interim interdict restrains the complainant from making exactly these disclosures. A court order that overrides a statutory protection enacted by Parliament is constitutionally suspect on its face, particularly where the order was obtained by concealment.
Rule nisi extended to 1 March 2027 – effective final order	The rule nisi was extended on 28 May 2026 to 1 March 2027. This is a period of almost one year from the original grant. The complainant has been silenced by an interim order, obtained by concealment, for a period that effectively constitutes a final restraint without a final hearing. The chilling effect on parliamentary communication alone justifies urgent judicial intervention.
Biowatch violation – Scale C costs threatened against destitute litigant	Scale C costs are the highest scale of legal costs in South African litigation. Threatening Scale C costs against a self-represented, destitute, homeless litigant who is asserting constitutional rights (including PDA Section 9 and

FINDING	EVIDENCE / SOURCE
	Constitution Section 56) is prima facie inconsistent with the Biowatch principle.
Apparent perjury across court divisions	In the Western Cape High Court, the Group CEO of AfroCentric acknowledged the existence of Labour Court Case 2026-077546. In the Labour Court, the same corporate group filed a Statement of Response stating: “The Defendants are not aware of any further claim with case number 2026-077546.” This is a directly contradictory sworn statement across two court divisions. It is a potential perjury referral.
Section 162 application struck (2026-066952)	Filed on 19 March 2026 against four named AfroCentric directors and the Group Company Secretary. Struck from the roll on 25 March 2026 with Scale C costs after an unauthorised advocate appeared. Grounds for Rule 42 rescission and a Judicial Conduct Tribunal complaint have been identified. The Biowatch principle is the constitutional basis for challenging the costs order.

17.2 Labour Court – Portal Obstruction and Default Judgment Delay

FINDING	EVIDENCE / SOURCE
Eight days of portal obstruction (13–20 March 2026)	The complainant’s first attempt to file with the Labour Court resulted in a deleted user profile, ignored emails to the call centre, and ultimately a rejection notice carrying the reason: “good day you put date.” This is not a technical error. It is an administrative failure by a court system that cannot process urgent applications from self-represented litigants with basic competence. Access to justice under Section 34 of the Constitution was denied for eight consecutive days.
Default judgment application unallocated for 6+ weeks	The default judgment application on Case 2026-077546 (uncontested BCEA salary deduction claim of R14,811.77) was filed on 30 April 2026. As at June 2026 – more than six weeks later – no hearing date has been allocated, no Registrar response has been received, and the matter appears to be in administrative limbo. There is no contestation. There is no complexity. The only explanation for the delay is administrative failure.

17.3 Registrar Misconduct and POPIA Breach

FINDING	EVIDENCE / SOURCE
Court recording withheld for 2+ months	The High Court Registrar has failed to provide the audio recording of the Section 162 hearing (Case 2026-066952), which the complainant requires in order to prepare a Rule 42 rescission application. The transcription service (Inlexso) advised that it had no backup system during its officer’s bereavement leave. The recording has been inaccessible for over two

FINDING	EVIDENCE / SOURCE
	months. Without it, the rescission application cannot be brought.
POPIA breach	The High Court Registrar transmitted documents from a third party's case file to the complainant – an unauthorised disclosure of another person's personal information, in breach of the Protection of Personal Information Act 4 of 2013. This breach has not been referred to the Information Regulator and has not been acknowledged by the Registrar.
Misrepresentation of filing records (EH/JO/2026/003)	The complainant identified an alleged misrepresentation of filing records in communications with the Director of Court Operations – documented in formal escalation memorandum EH/JO/2026/003, addressed to the Judge Presidents of the Labour Court and the Western Cape High Court.
CONCLUSION: The justice system has not provided a remedy. It has become another layer of obstruction. A gag order obtained by concealment, a portal that rejects urgent applications with administratively incompetent messages, a six-week wait for an uncontested default judgment, a POPIA breach without acknowledgment, and a two-month delay in providing a mandatory court recording: together, these failures constitute a systemic denial of access to justice under Section 34 of the Constitution.	

PART EIGHTEEN: CORPORATE MISREPRESENTATION – AFROCENTRIC’S FALSE INTEGRATED ANNUAL REPORT 2025

AfroCentric Investment Corporation Limited is a JSE-listed company (JSE: ACT). Its Integrated Annual Report 2025 is a document intended to be relied upon by investors, shareholders, government counterparties, regulatory bodies, and the public. Its contents are representations to the market. Where those representations are materially false or misleading, they constitute a basis for regulatory enforcement by the JSE and the FSCA, and potentially a basis for liability under the Financial Markets Act.

FINDING	EVIDENCE / SOURCE
AfroCentric claimed: “Zero-tolerance approach to unethical conduct” and “whistleblower protection.”	Documented truth: The whistleblower was retaliated against from October 2025 to December 2025; was constructively dismissed; was silenced by a gag order obtained by concealment; was left destitute and homeless; and was deregistered as a nurse. The employer’s Chief Risk Officer actively suppressed the Priority 1 Material Risk Report. No whistleblower protection mechanism was triggered at any level of the organisation.
AfroCentric claimed: “No material ESG-related incidents, regulatory breaches, fines or penalties.”	Documented truth: Priority 1 clinical risk (Gexus-DMS gap); unlawful 40.47% salary deduction violating BCEA Section 34(2); active B-BBEE Commission investigation (Case 5/02/2026, potential fine of R580 million); active criminal case (CAS 705/3/2026); Section 162 director delinquency application (Case 2026-066952) against four named directors and the Group Company Secretary.
AfroCentric claimed: No material litigation disclosed.	Documented truth: Four concurrent court proceedings; B-BBEE investigation; criminal complaint; regulatory complaints across eleven statutory bodies. The Section 162 application alone, filed on 19 March 2026, is material – it targets named directors and carries governance consequences. The company confirmed it received notice of Case 2026-083299 (Labour Court, constructive dismissal) in its High Court pleadings while simultaneously swearing in Labour Court that it was not aware of Case 2026-077546.
AfroCentric claimed: “Both platforms fully integrated” (Gexus and DMS).	Documented truth: The internal Root Cause Analysis, generated by Medscheme’s own systems, confirmed: “miscommunication between Gexus and DMS systems ... system integration gaps.” This is a direct and irreconcilable contradiction between the public report and the internal findings.
AfroCentric claimed: B-BBEE Management Control score 14.67/19 – Level 1 status.	Documented truth: Title suppression and fronting confirmed in writing by Cheryl-Dawn Modern (19 January 2026). B-BBEE Commission Case 5/02/2026 active. The Management Control score on which the Level 1 rating partly depends was calculated using the credentials of a professional whose title was suppressed and who was not paid in accordance with those credentials.

FINDING	EVIDENCE / SOURCE
CONCLUSION: AfroCentric’s Integrated Annual Report 2025 is a materially false document. It does not merely omit; it actively misrepresents. Every investor who relied on this report was misled. Every government counterparty who relied on this report was misled. The JSE allowed it to stand. The FSCA went silent. This is a corporate cover-up facilitated by regulatory failure.	

PART NINETEEN: THE JSE AND FSCA – REGULATORY COMPLICITY AND CHILLING EFFECT

The Johannesburg Stock Exchange (JSE) and the Financial Sector Conduct Authority (FSCA) are the twin pillars of capital market accountability in South Africa. The JSE enforces its Listings Requirements, which require listed companies to disclose material information to the market on a timely basis. The FSCA enforces the Financial Markets Act 19 of 2012 and monitors market conduct. When confronted with evidence of material non-disclosure by a JSE-listed company, both bodies declined to act.

FINDING	EVIDENCE / SOURCE
JSE declined jurisdiction (6 March 2026)	JSE letter of 6 March 2026 (Ref: JvS/166792) declined jurisdiction over the material non-disclosure complaint, referring the complainant to the CCMA and Labour Court. This is a jurisdictional error: the complainant's complaint to the JSE was not about a labour dispute. It was about a listed company's failure to disclose material information to the market. The JSE's Listings Requirements govern exactly this obligation.
JSE confirmed no action (11 May 2026)	Following escalation, the JSE confirmed in its letter of 11 May 2026 that it would take no enforcement action in respect of AfroCentric's Integrated Annual Report 2025 or the non-disclosed litigation and investigations.
Section 162 application not required to be disclosed	The JSE did not require AfroCentric to disclose the Section 162 director delinquency application (filed 19 March 2026) via SENS. A court application targeting named directors of a listed company for delinquency declarations is, by any reasonable assessment, material information for investors.
B-BBEE investigation not required to be disclosed	The JSE did not require disclosure of B-BBEE Commission Case 5/02/2026, which carries a potential fine of 10% of annual turnover – estimated at R580 million. A potential nine-figure fine against a JSE-listed company is manifestly material information.
FSCA went silent (Ref: 1-370127)	The FSCA acknowledged receipt of the complainant's complaint with a reference number (1-370127) and then provided no substantive response. No investigation was opened. No acknowledgment of the substantive allegations was provided.
Chilling effect on market integrity	The combined effect of the JSE's and FSCA's non-enforcement is a clear market signal: JSE-listed companies may omit Section 162 applications, active B-BBEE investigations, criminal cases, and clinical risk events from their integrated reports without regulatory consequence. This chilling effect on disclosure will directly infect the governance of companies that will be NHI-accredited providers and administrators.
CONCLUSION: The JSE and FSCA have created a regulatory vacuum in which material non-disclosure by a JSE-listed government contractor carries no enforcement consequence. This is not merely a failure of capital market accountability. It is a structural enabler of the same pattern of corporate misrepresentation that will infect the NHI if governance architecture is not strengthened before implementation.	

PART TWENTY: BAD FAITH LITIGATION AND ATTRITION – THE CORPORATE PLAYBOOK

The employer's litigation conduct – documented across four court proceedings and multiple regulatory engagements – follows a recognisable corporate attrition playbook: respond with procedural defects; introduce perjury by omission; raise retroactive service disputes; propose non-compliant dates; communicate with contempt; offer settlement with conditions designed to suppress constitutional rights; and then go silent. Each individual tactic creates delay, uncertainty, and cost. Together, they are designed to exhaust a destitute, self-represented litigant until he capitulates.

FINDING	EVIDENCE / SOURCE
Defective initial Statement of Response	The Statement of Response in Labour Court Case 2026-083299 was filed on behalf of a non-cited entity and without the required oath, introducing procedural defects that required additional filings from the complainant and additional delay.
Perjurious denial of knowledge (19 May 2026)	The Statement of Response states: "The Defendants are not aware of any further claim with case number 2026-077546." In the Western Cape High Court proceedings in Case 2026-086906, the Group CEO of AfroCentric acknowledged the Labour Court claims. The contradictory sworn statements across two court divisions constitute a potential perjury referral to the NPA.
Retroactive service dispute (1 June 2026)	Vijay Makenjee of Deneys Reitz raised a service dispute via email on 1 June 2026, after pleadings had already been filed and the matter was ready to proceed to pre-trial. This is a retroactive procedural objection designed to restart the clock and impose additional cost and uncertainty on the complainant.
Non-compliant pre-trial date (2 June 2026)	Deneys Reitz proposed a pre-trial conference date of 22 June 2026 – a date outside the statutory window under Labour Court Rule 22 (2024). The complainant filed a Unilateral Pre-Trial Minute on 11 June 2026 in response to the employer's non-engagement.
"Good luck to you then" (2 June 2026)	Vijay Makenjee responded to a statutory compliance request from the complainant with the words: "Good luck to you then." This communication is not an oversight. It is a deliberate display of contempt for the complainant's legal rights, designed to signal that the employer has no intention of engaging in good faith.
Parliamentary contempt threat (4 June 2026)	Deneys Reitz Inc.'s "without prejudice" letter of 4 June 2026 threatened the complainant with contempt of court if he approached Parliament. This is an attempt to suppress a constitutional right – the right to petition Parliament under Section 56 of the Constitution. It is itself potentially contemptuous of Parliament. It warrants referral to the Legal Practice Council.
Silence after settlement overture	Having extended the "without prejudice" overture, Deneys Reitz went silent for over a week after the complainant received it. This is a deliberate litigation tactic: the overture creates uncertainty and delay; the silence

FINDING	EVIDENCE / SOURCE
	compounds the pressure. Neither the overture nor the silence represents a genuine attempt to resolve the dispute.
CONCLUSION: The employer is not seeking justice. It is deploying a structured and deliberate attrition strategy designed to exhaust a destitute, self-represented, homeless whistleblower until he has no resources left to continue. The conduct of Deneys Reitz Inc. – including the threat to hold a citizen in contempt for petitioning Parliament – warrants formal referral to the Legal Practice Council and, where applicable, to the National Prosecuting Authority.	

PART TWENTY-ONE: THE NHI GOVERNANCE GAP – LINKING FINDINGS TO THE NATIONAL HEALTH INSURANCE

This report has documented fifteen institutional failures across the regulatory, judicial, and corporate governance architecture of South Africa. Each failure is a primary source finding, supported by documentary evidence. Together, they are not merely the story of one whistleblower's destruction. They are a forensic prediction of what will happen inside the NHI Fund unless the governance architecture is strengthened before implementation.

The accompanying article – “Building the Watchdog Before the House Burns Down: Forensic Governance Architecture for NHI Implementation” – identifies three structural vulnerabilities in the NHI Act 20 of 2023. This section links each vulnerability directly to the evidence in this report.

Vulnerability 1: The Accreditation Bottleneck (NHI Act Section 39)

The NHI Act requires all providers to be accredited by the Office of Health Standards Compliance (OHSC). The OHSC was documented by the Auditor-General as capacity-constrained before the NHI was enacted. When financial pressure meets an institutional capacity constraint in an accreditation system, the predictable outcomes are ghost accreditation (certification issued without genuine compliance assessment) and fronting (accredited entities serving as conduits for unaccredited providers). Both outcomes divert public funds to unqualified providers without clinical accountability.

This report provides direct evidence that both outcomes already occur in the existing system. The B-BBEE fronting complaint (Case 5/02/2026) documents exactly the dynamic that will recur at NHI scale: a Black NQF Level 8 professional's credentials are used for compliance scorecard purposes while the professional himself is denied the title, compensation, and management authority associated with those credentials. The B-BBEE Commission, the institution that would be expected to investigate NHI accreditation fronting, declined to investigate, refused its Section 13K subpoena powers, and classified fronting as a labour dispute. If it cannot investigate fronting in the existing system, it will not investigate it under the NHI.

- Required reform: Mandatory Forensic Governance Impact Assessments before each phase of NHI accreditation rollout, independently reviewed and tabled in Parliament. These assessments must specifically audit the OHSC's capacity against the accreditation volume required at each phase, identify fronting risk vectors, and establish monitoring protocols for ghost accreditation.

Vulnerability 2: The One-Sentence Fraud Architecture (NHI Act Section 40)

Section 40 of the NHI Act provides that “the information architecture must include a fraud and risk management mechanism.” Eleven words. For a system that will process millions of clinical and financial transactions annually, determine the health benefits of 60 million people, and become the single largest concentration of health care funds in South African history.

This report documents what happens when a health care information platform has a structural integration gap and there is no independent fraud detection architecture to identify or report it. The Gexus-DMS integration gap generated false non-adherence data. The false data was used to generate clinical compliance letters. The RCA confirmed the structural design flaw. The CMS declined jurisdiction twice. The employer's Annual Report said both platforms were “fully integrated.” No independent audit caught the contradiction. The market regulator did not require disclosure. The gap remains unaddressed.

At NHI scale, this pattern – structural data errors generating false clinical flags, hidden from regulators, undisclosed to the market, suppressed by internal risk management structures – would result in systemic misallocation of clinical resources, inflated claims data, and potentially billions in fraudulent or erroneous payments.

- Required reform: A mandatory AI Audit Protocol for the NHI’s information platform – independent algorithmic conformity assessment, mandatory transparency obligations for all platform operators, human oversight requirements for all automated clinical decision-support systems, and annual algorithmic audits published to Parliament and the public.

Vulnerability 3: The Undefined Investigation Unit (NHI Act Section 20(2)(g))

Section 20(2)(g) of the NHI Act requires the CEO to establish a “Risk Management and Fraud Prevention Investigation Unit.” The Act provides no definition of the unit’s professional competency requirements, no independence safeguards (the unit reports to the CEO, who reports to the Board, whose members may themselves be implicated in the fraud or risk being reported), no mandatory escalation protocols to the SIU, Hawks, or NPA, and no consequences for suppression of risk reports.

The Zondo Commission documented this exact failure pattern across multiple state-owned entities: risk management units with broad mandates, no independence safeguards, and no escalation protocols that became instruments of suppression rather than disclosure. This report provides direct evidence of exactly this pattern in the existing health care administration system. The Chief Risk Officer of AfroCentric (Monwabisi Kula) – the person whose unit should have escalated the Priority 1 Material Risk Report to the relevant authorities – instead instructed the complainant to stop emailing the Board. This is suppression. It is documented. And it occurred in a company that holds a government administration contract for 1.9 million public servants.

- Required reform: A Statutory Forensic Governance Protocol for the NHI Fraud Investigation Unit, establishing: mandatory professional competency standards (forensic governance certification, minimum years of relevant experience); independence safeguards (direct reporting line to the Board, bypassing executive management, with an independent audit function); mandatory escalation protocols to the SIU, Hawks, and NPA for all Priority 1 and Priority 2 findings; criminal penalties for suppression of escalated risk reports; and annual public reporting to Parliament.

The PPE procurement crisis cost South Africa R14.3 billion in a single procurement emergency. The NHI Fund will manage R200 billion annually in perpetuity. The same institutional failures that enabled the PPE crisis – weak accreditation, absent fraud architecture, undefined oversight units – are already present and already causing harm in the healthcare administration system. Parliament must implement these three reforms before the NHI becomes operational. Building the watchdog after the house burns down is not governance. It is an autopsy.

PART TWENTY-TWO: STATUS OF PARALLEL PROCEEDINGS (LITIGATION UPDATE)

Current as at June 2026. This section provides a consolidated status table of all active proceedings referenced in this report.

Case No. / Ref	Nature of Proceeding / Body	Status as at June 2026 Delay
2026-086906 (Western Cape High Court)	Interdict application by employer (gag order). Rule nisi extended to 1 March 2027.	ACTIVE – silences whistleblower for 11 months. Final hearing pending semi-urgent roll. Obtained by concealment of answering affidavit.
2026-066952 (Western Cape High Court)	Section 162 director delinquency application. Struck from roll 25 March 2026 with Scale C costs.	RESCISSION PENDING – Registrar has not provided court audio recording (2+ months). Inlexso had no backup during bereavement leave.
2026-083299 (Labour Court)	Constructive dismissal / automatically unfair dismissal (PDA retaliation). Replication filed. Unilateral Pre-Trial Minute filed 11 June 2026.	ACTIVE – awaiting court direction on pre-trial. Employer's Statement of Response was defective (non-cited entity, no oath). "Good luck to you then" non-engagement.
2026-077546 (Labour Court)	BCEA Section 77A – unlawful salary deduction (R14,811.77). Default judgment application filed 30 April 2026.	STAGNANT – unallocated for 6+ weeks. Registrar non-responsive. No hearing date.
CAS 705/3/2026 (SAPS Woodstock)	Criminal case – UIF fraud and false compliance representations.	STAGNANT – no detective assigned; no investigation in 3+ months. Internal Delft complaint closed without addressing false representation.
5/02/2026 (B-BBEE Commission)	Fronting complaint – title suppression of NQF Level 8 Black professional.	CLOSED (Notice of Non-Investigation, 30 March 2026). Commission refused Section 13K subpoena powers. Reclassified as labour dispute.
1-370127 (FSCA)	Governance failure and market conduct complaint.	SILENT – acknowledged receipt; no substantive response; no investigation opened.
JvS/166792 (JSE Issuer Regulation)	Non-disclosure of material litigation and investigations in Integrated Annual Report 2025.	CLOSED (JSE confirmation, 11 May 2026). JSE declined to enforce disclosure or require corrective SENS announcement.
SAHRC Western Cape	Constitutional rights violations (Sections 10, 26, 27, 33 of the Constitution).	SILENT – acknowledged February 2026; 13 March 2026 update ignored; no investigation opened; no own-initiative investigation triggered.
SANC/MEDSCHEME/MATTHEE/20260105	Professional misconduct complaint against two registered nurses employed by Medscheme.	NOT ACKNOWLEDGED – filed 10 February 2026; not acknowledged, assigned, or processed as at June 2026.

Consolidated Delay Summary

Proceeding	Delay and Cause
Labour Court default judgment (2026-077546)	45+ days unallocated. Registrar unresponsive. No hearing date.
Labour Court pre-trial (2026-083299)	14+ days beyond statutory window. Employer proposed non-compliant date; non-engagement; "Good luck to you then."
High Court final hearing (2026-086906)	11 months (extended to March 2027). Court accepted employer's request for semi-urgent roll without substantive hearing.
SAPS criminal investigation (CAS 705/3/2026)	3+ months since registration. No detective assigned.
High Court audio recording (for rescission)	2+ months. Registrar non-responsive. Inlexso no backup system during bereavement leave.
SANC complaint (SANC/MEDSCHEME/MATTHEE/20260105)	4+ months. Not acknowledged.

PART TWENTY-THREE: CONCLUSION – WHY PARLIAMENT MUST ACT NOW

The whistleblower in this case did everything the law asked of him. He identified a system error. He refused to lie to a patient. He made a protected disclosure. He exhausted internal remedies. He escalated to the CEO. He escalated to the Board. He approached every regulator. He approached every law enforcement body. He filed court applications. He appointed no lawyer – because he cannot afford one. He is sleeping without running water. And now he has come to Parliament.

Fifteen institutions failed him. The following table records each failure and its direct consequence for the complainant, for GEMS beneficiaries, and for the NHI governance architecture.

FINDING	EVIDENCE / SOURCE
CMS	Declined jurisdiction twice. Patient safety risk unaddressed. NHI accreditation regulator already captured.
HPCSA	Silent on core ethical question. No guidance for health professionals across South Africa.
SANC	Complaint unacknowledged. Deregistered the complainant. Nursing ethics unenforced.
SAPC	Jurisdiction declined. Pharmacist accountability avoided.
GEMS	No response to two protected disclosures. Principal scheme complicit. 1.9 million members remain exposed.
DEL	File closed with error of law. UI-19 not corrected. UIF contributions not remitted. Complainant without social security.
CCMA	Biased conciliation. PSC cover-up. No fair dispute resolution for the most vulnerable litigants.
Labour Court Registry	Portal obstruction. Default judgment stagnant. Section 34 constitutional right denied.
SAPS	Criminal case stagnant. False station commander representation. CAS 705/3/2026 uninvestigated.
JSE	Material non-disclosure allowed to stand. Market misled. Corporate cover-up facilitated.
B-BBEE Commission	Fronting complaint declined. Section 13K powers refused. Fronting inside a government contractor uninvestigated.
SAHRC	Acknowledged seriousness; went silent. Sections 10, 26, 27, 33 violations unaddressed.
High Court	Ex parte interdict by concealment. Gag order for 11 months. Protected whistleblower silenced.
TWBH / Schroter Attorneys	Flawed legal assessment. Abandoned a destitute whistleblower to self-represent in nine proceedings.
FSCA	Acknowledged and went silent. No market conduct investigation.

This is not a series of isolated failures. It is a pattern. And the pattern is forensic prediction. If fifteen institutions – each with a clear mandate, each notified, each provided with primary source evidence – failed to provide a remedy to a protected whistleblower in the existing healthcare administration system, then those same fifteen institutions will fail the first whistleblower who reports corruption or clinical failure inside the NHI Fund. And the money that is lost inside the NHI Fund will not be R14,811.77. It will be measured in billions.

Parliament has a choice. It can act now – summon the failed institutions, demand accountability, close the governance gaps in the NHI Act, and build the watchdog before the house burns down. Or it can wait – and explain to the South African people, when the first NHI whistleblower is destroyed and the first R200 billion is lost, why it failed to act when the evidence was placed before it.

The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke. Parliament must act before the NHI becomes the fire.

PART TWENTY-FOUR: RECOMMENDATIONS TO PARLIAMENT

A. Summons and Accountability Hearings

1. Summon the Registrar of the Council for Medical Schemes to explain in writing and before the Portfolio Committee on Health why the Gexus-DMS integration gap was not investigated under MSA Sections 7, 47, 58, and 44, and to provide a written explanation for the differential treatment of Discovery Health (jurisdiction exercised, January 2026) vs Medscheme/GEMS (jurisdiction declined twice).
2. Summon the Registrar of the South African Nursing Council to explain why complaint SANC/MEDSCHEME/MATTHEE/20260105 (filed 10 February 2026) was not acknowledged, assigned, or processed over four months, and to explain the basis on which the complainant was deregistered for non-payment of fees caused by the employer's unlawful deduction.
3. Summon the Chairpersons of the Health Professions Council of South Africa and the South African Pharmacy Council to appear jointly before the Portfolio Committee on Health and issue a formal written ruling on the core ethical question: whether a registered health professional may refuse an employer's instruction to act on clinical data known to be false when that action could harm a patient.
4. Summon the Director-General of the Department of Employment and Labour to explain why the BCEA Section 34(2) 25% deduction cap was not enforced, why the false UI-19 was not corrected, and why the investigation file was closed with an error of law. The Director-General must bring with him or her: the Compliance Order issued (or an explanation for why no Compliance Order was issued); the UI-19 correction (or an explanation for why it was not corrected); and the legal opinion relied upon for the jurisdictional misdirection to the CCMA.
5. Summon the B-BBEE Commissioner to explain the legal basis for classifying the complainant's fronting complaint as a labour dispute, and to explain why Section 13K subpoena powers were not deployed. The Commissioner must bring the full case file for Case 5/02/2026 and the legal opinion relied upon for the Notice of Non-Investigation.
6. Summon the Chairperson of the South African Human Rights Commission to explain why, after acknowledging the seriousness of the complainant's complaint in February 2026, the SAHRC did not open a formal investigation, did not respond to the 13 March 2026 update confirming homelessness, and did not trigger its own-initiative investigation power under SAHRC Act Section 14.
7. Summon the National Commissioner of the South African Police Service to explain why CAS 705/3/2026 has not been investigated for three months, why no detective has been assigned, and what action has been taken against the Delft Police Station officer who falsely claimed to be station commander. The National Commissioner must bring the full case file and the internal complaint outcome.
8. Summon the Head of Issuer Regulation of the JSE and the Commissioner of the Financial Sector Conduct Authority to explain why AfroCentric's Integrated Annual Report 2025 – containing material misrepresentations and omissions – was allowed to stand without enforcement action or a required SENS announcement.
9. Request the Judge President of the Western Cape High Court to commission an internal investigation into the concealment of the complainant's answering affidavit at the ex parte hearing of 16 April 2026, and into the POPIA breach by the High Court Registrar.
10. Request the Judge President of the Labour Court to commission an internal investigation into the eight-day portal obstruction (March 2026), the six-week stagnation of the default judgment application in Case 2026-077546, and the Registrar's non-responsiveness to follow-up correspondence.

B. Legislative Reforms – NHI Governance Architecture

11. Mandate Forensic Governance Impact Assessments (FGIAs) before each phase of NHI accreditation rollout. Each FGIA must be independently conducted, must audit the OHSC's accreditation capacity against the volume required at each phase, must identify fronting and ghost accreditation risk vectors, and must be tabled in Parliament before the phase proceeds.
12. Enact a mandatory AI Audit Protocol as a Schedule to the NHI Act or as standalone legislation, requiring all operators of clinical information platforms used by the NHI Fund to submit to independent algorithmic conformity assessments, maintain transparency logs, implement human oversight for automated clinical decisions, and publish annual audit reports to Parliament.
13. Enact a Statutory Forensic Governance Protocol for the NHI Fraud Investigation Unit, establishing: mandatory professional competency standards (minimum forensic governance certification and five years' relevant experience); an independent reporting line directly to the Board (bypassing executive management); mandatory escalation protocols to the SIU, Hawks, and NPA for all Priority 1 and Priority 2 risk findings; criminal penalties for suppression of escalated risk reports by any officer; and annual public reporting to Parliament.

C. Whistleblower Protection – PDA Amendments

14. Amend Section 4 of the Protected Disclosures Act to require employers, within five business days of a finding of prima facie retaliatory deduction, to restore the deducted amount pending the outcome of court proceedings. The finding must be made by the CCMA or Labour Court on an urgent basis, without requiring full adjudication of the merits.
15. Establish a dedicated Whistleblower Protection Authority as a standalone statutory body, with the power to: receive protected disclosures; monitor the regulatory responses of all bodies notified of the disclosure; apply for urgent interdicts against retaliation on behalf of the whistleblower in the Labour Court or High Court; and refer matters directly to the NPA. The WPA must have an independent budget and must report annually to Parliament.
16. Amend the Protected Disclosures Act to impose criminal penalties on officials of regulatory bodies who decline jurisdiction without legal basis, delay processing of whistleblower complaints beyond statutory timeframes, or who make false statements in regulatory responses to parliamentary oversight bodies.

D. Referrals for Prosecution and Professional Discipline

17. Refer Deneys Reitz Inc. to the Legal Practice Council for: (a) threatening the complainant with contempt of court for petitioning Parliament under Section 56 of the Constitution (letter of 4 June 2026); (b) concealing the complainant's answering affidavit from the High Court at the ex parte hearing of 16 April 2026 (fraud on the court); and (c) the pattern of bad faith settlement tactics documented in Part Twenty.
18. Refer Vijay Makenjee of Deneys Reitz to the Legal Practice Council and to the National Prosecuting Authority for: the "Good luck to you then" communication (contemptuous conduct in the course of legal proceedings); and the apparent perjury in the Labour Court Statement of Response (denial of knowledge of Case 2026-077546 contradicted by the High Court pleadings).

19. Refer the Delft Police Station officer who falsely claimed to be station commander (23 February 2026, audio-recorded) to the Independent Police Investigative Directorate for investigation under the IPID Act 1 of 2011.
20. Refer the apparent perjury in the Labour Court Statement of Response (denial of knowledge of Case 2026-077546 contradicted by the Group CEO's High Court pleadings) to the National Prosecuting Authority.
21. Refer the conduct of CCMA Commissioner Ntabiseng Ngwane and Provincial Senior Commissioner Vusumzi Landu to the Public Protector for investigation of institutional maladministration and deliberate cover-up of commissioner misconduct.

PART TWENTY-FIVE: FINAL PRAYER FOR RELIEF

Honourable Speaker, Members of Parliament,

The whistleblower who has brought this matter before you is a Specialist Registered Nurse. He holds a BTech in General Nursing and a Postgraduate Diploma in Health Services Management, awarded with distinction. He identified a systemic clinical risk. He refused to lie to a 75-year-old patient. He made a protected disclosure. He escalated to every level of the institution. He approached every regulator, every law enforcement body, and every court available to him.

He is now homeless. He has no running water. He has been deregistered by the South African Nursing Council because he could not pay R870 – an amount that he does not have because his employer deducted R14,811.77 from his salary without consent, without a court order, and without the written agreement required by the Basic Conditions of Employment Act. He is self-represented in nine concurrent proceedings. He has been silenced by a court order obtained by the concealment of his own answering affidavit. He has been threatened with contempt of court for petitioning Parliament.

He has been told, in effect, by fifteen institutions: your disclosure does not matter; your rights do not matter; your dignity does not matter; your water does not matter; your registration does not matter; your housing does not matter. None of it matters.

Parliament is the last door.

The prayer for relief is not complex. It is this: exercise the constitutional oversight power that every one of the fifteen institutions in this report abdicated. Summon those who failed. Demand accountability. Close the governance gaps that will allow the same pattern to repeat inside the R200 billion NHI Fund. Protect future whistleblowers from induced destitution. And acknowledge – in the most powerful forum available in this democracy – that what happened to this nurse was wrong, that it was documented, and that Parliament saw it and acted.

Not because the personal damages will repair the harm. They will not. Not because a parliamentary resolution will restore what has been lost. It will not. But because the alternative is a precedent that will echo through every clinical environment in South Africa in the years to come: that the person who refuses to lie to a 75-year-old patient will be destroyed for it, and the institutions that are supposed to prevent that destruction will look the other way.

The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke. Parliament must act before the NHI becomes the fire.

Prepared by: EthicHawks Forensic Governance & Compliance Consultancy

On behalf of: Sr. Wilbur William Matthee, Specialist Registered Nurse (NQF Level 8)

Contact: info@ethichawks.co.za | 069 635 1435

Date: June 2026

EPILOGUE – THE HUMAN FACE BEHIND THIS REPORT

To the Honourable Speaker and Members of Parliament,

I close this report not with a legal argument, not with a citation, not with a schedule of damages, and not with a prayer for relief. I close it with a name – or rather, the absence of one. And a case number.

GEMS 000655789 Dependent 02.

I do not know her name. I was never supposed to know it – she is a GEMS beneficiary, protected by the same confidentiality obligations that governed every patient interaction in my clinical work. I know that she is an elderly widow. I know that her daughter wrote to Medscheme on her behalf. I know the words her daughter used, because I read the correspondence myself: “I request that you get your house in order and update your records. Stop confusing old lady.”

She had been receiving false non-adherence letters. She collected her cholesterol medication every month. She did everything right. And the system failed her – not because of clinical negligence, but because of a structural integration gap between two data platforms that no one in that organisation was willing to acknowledge, investigate, or disclose. She was being accused, in writing, of not taking her medication. And she was doing exactly what her doctor prescribed.

On 23 October 2025, I was told to contact her. To interrogate an elderly widow about medication she was taking correctly, using data that I knew to be false. I refused.

That refusal is what this report is about. Not the unlawful deduction. Not the constructive dismissal. Not the nine concurrent proceedings. Not the interim interdict obtained by concealment. Not the fifteen institutions that failed me. Those are the consequences of the refusal. But the refusal itself was simple: I am a nurse. A nurse’s first duty is to the patient. And I refused to use false data to harm an elderly patient.

I also think of my mother. She is not here to read this report. She cannot see what has happened to me, and I am grateful for that. But everything I have done since 23 October 2025 – every affidavit, every regulatory complaint, every hour of legal research in a building without running water – I have done because she taught me that a nurse’s first duty is to the patient. Not to the employer. Not to the system. Not to the profit margin. To the patient.

A nurse who lies to a patient is not a nurse. A system that punishes the nurse who refuses to lie is not a healthcare system. It is a conspiracy against the vulnerable.

Parliament has before it more than 500 pages of forensic evidence. Fifteen failed institutions. A corporate cover-up. A court order obtained by the concealment of an affidavit. A whistleblower who has been reduced to living without running water in a city that produces two of the world’s top 100 universities.

But behind every page of this report is a 75-year-old widow who deserved to be told the truth about her medication. And behind every signature on this report is a nurse who refused to lie to her.

The question before Parliament is not whether I was constructively dismissed. That question is before the Labour Court. The question before Parliament is not whether Medscheme breached the BCEA. That question is before the Labour Court too. The question before Parliament is this: are the institutions that failed me the same institutions that will fail the next whistleblower inside the NHI Fund? And if so, what are you going to do about it before it happens?

I have done everything the law asked of me. I have filed every form. I have knocked on every door. I have been to every institution that is supposed to exist to protect people like me.

This is the last door. I place my trust in Parliament.

Wilbur William Matthee

RN | BTech General Nursing | PGDip Health Services Management (NQF 8)

Founder – EthicHawks Forensic Governance & Compliance Consultancy

Cape Town, South Africa | June 2026

“The house is not yet burning. But the fire alarm has been ringing for over a year. This report is the evidence of the smoke. Parliament must act before the NHI becomes the fire.”

INDEX OF ANNEXURES – CONSOLIDATED EVIDENCE BUNDLE

All documents listed below are primary source documents generated by the employer, regulatory bodies, the courts, or the complainant during the events described in this report. The full bundle of 526 source documents is available to Parliament upon request. The complainant requests that a parliamentary legal officer or independent forensic investigator be appointed to verify the authenticity of the evidence bundle.

Ref	Description	Date / Source
A1	Interview invitation – “Specialist: Care Manager” role (3 invitations across 3 interview rounds).	January 2024 – AfroCentric / Medscheme
A2	Employment contract – “Care Coordinator” (Grade B4M1). Contradicts A1 in title and grade.	9 April 2024 – AfroCentric / Medscheme
A3	First Aider roster – management of 13+ staff across 3 floors. Evidence of actual role scope.	25 July 2025 – Complainant
B1	Dr Jacques Malan psychiatric letter – severe depression and PTSD (occupational causation). Formal diagnosis.	16 May 2025 – Dr Jacques Malan FCPSYCH(SA)
C1	“Wilma” Microsoft Teams message – gender-identity harassment. Sender and content documented.	23 September 2025 – Medscheme systems
C2	Formal discrimination complaint (11 pages). Submitted to employer the same day as the harassment.	25 September 2025 – Complainant
C3	Retaliatory unpaid leave entry in payroll system – loaded same day as the formal discrimination complaint.	29 September 2025 – Medscheme payroll
D1	Gexus-DMS Root Cause Analysis (RCA) – confirms integration gap and structural design flaw.	23 October 2025 – Medscheme internal investigation
D2	Protected disclosure email – complainant refuses to contact patient on false data. Formal PDA disclosure.	23 October 2025 – Complainant
D3	Manager’s response to protected disclosure: “I am not in agreement with your feedback.”	23 October 2025 – Mary-Ann Jones
E1	Recorded meeting transcript – manager admits: “You made a case of harassment against me, so I can make a case against you.” Audio available.	31 October 2025 – Complainant (audio recording)
F1	Executive escalation to CEO (32 attachments). Requests paid leave accommodation and settlement discussions.	4 December 2025 – Complainant
G1	Priority 1 Material Risk Report to Board (38 attachments). Includes Section 60 EEA personal liability warning.	9 December 2025 – Complainant
H1	Employer’s CCMA opposing affidavit – clinically diagnosed depression described as a “cynical excuse.” Potential perjury.	10 December 2025 – Xolile Manyati / AfroCentric
H2	Resignation letter – explicitly invokes LRA Section 186(1)(e): automatically unfair constructive dismissal.	11 December 2025 – Complainant
I1	CRO suppression email – Monwabisi Kula instructs complainant to stop emailing directors.	18 December 2025 – Monwabisi Kula (CRO, AfroCentric)
J1	CMS declination letter (first) – refers to HPCSA without jurisdictional analysis.	4 February 2026 – CMS

Ref	Description	Date / Source
J2	SAHRC response – acknowledges seriousness, redirects to CCMA and Public Protector.	10 February 2026 – SAHRC
J3	SANC formal complaint (SANC/MEDSCHEME/MATTHEE/20260105) – not acknowledged to date.	10 February 2026 – Complainant
K1	JSE declination letter (Ref: JvS/166792) – refers to CCMA and Labour Court.	6 March 2026 – JSE
K2	Labour Court portal rejection notice – reason given: “good day you put date.”	12 March 2026 – Labour Court registry
L1	Section 162 director delinquency application (Case 2026-066952) – targets four directors and Group Company Secretary.	19 March 2026 – Complainant
M1	High Court interim interdict – gag order granted ex parte after answering affidavit concealed.	16 April 2026 – Western Cape High Court
M2	Rule nisi extended to 1 March 2027 – complainant silenced for 11 months without a final hearing.	28 May 2026 – Western Cape High Court
N1	Employer’s Labour Court Statement of Response: “not aware” of Case 2026-077546. Apparent perjury.	19 May 2026 – Vijay Makenjee / AfroCentric
O1	“Good luck to you then” email – Vijay Makenjee’s response to statutory compliance request.	2 June 2026 – Vijay Makenjee, Deneys Reitz Inc.
P1	Deneys Reitz “without prejudice” letter – threatens contempt of court for petitioning Parliament.	4 June 2026 – Deneys Reitz Inc.
Q1	Unilateral Pre-Trial Minute – filed per Labour Court Rule 22 (2024) following employer’s non-engagement.	11 June 2026 – Complainant
R1	Final notice to employer – last deadline for settlement framework before parliamentary submission.	13 June 2026 – Complainant
S1	This Consolidated Forensic Oversight Report (all sections).	June 2026 – Complainant / EthicHawks

Audio Recordings Available Separately

Recording	Date	Content and Status
Recording 1	31 October 2025	Manager’s retaliatory admission: “You made a case of harassment against me, so I can make a case of harassment against you.” – Available; transcribed.
Recording 2	23 February 2026	Delft Police Station – officer falsely claims to be station commander and refuses to open criminal case. – Available; not yet formally transcribed.

All documents and recordings are available to Parliament upon request. The complainant requests that a parliamentary legal officer or independent forensic investigator appointed by Parliament verify the authenticity of the evidence bundle before any committee proceedings in which this report is tabled.

SIGNATURE BLOCK AND COMMISSIONER OF OATHS

I, the undersigned, WILBUR WILLIAM MATTHEE, declare under oath that the contents of this Consolidated Forensic Oversight Report are, to the best of my knowledge and belief, true and correct. I have personal knowledge of the facts set out herein, except where indicated to the contrary, and the documentary evidence referenced throughout is genuine and has not been materially altered or fabricated.

I am aware that making false statements in a submission to Parliament may constitute a criminal offence. I have not made any false statements. I have distinguished throughout between factual findings (supported by primary source documents) and legal opinions or submissions (clearly identified as such).

I make this report in good faith, under the protections of the Protected Disclosures Act 26 of 2000 (as a disclosure to Parliament as an appropriate authority under Section 8), and in the exercise of my constitutional right to petition Parliament under Section 56 of the Constitution of the Republic of South Africa, 1996. I invite Parliament to appoint an independent forensic investigator to verify the authenticity of the evidence bundle and to report to the relevant Portfolio Committees.

SIGNATURE:

Wilbur William Matthee

FULL NAME:	Wilbur William Matthee
SANC REGISTRATION NO.:	15831886 (currently lapsed – non-payment caused by employer’s unlawful deduction)
ID NUMBER:	890903 5151 087
CAPACITY:	Complainant / Protected Whistleblower / Self-Represented Litigant
CONTACT:	info@ethichawks.co.za 069 635 1435
ADDRESS:	15 Bellevue Street, Voorbrug, Cape Town, 7001 (currently homeless – service by email only)
DATE:	June 2026
PLACE:	Cape Town, South Africa

COMMISSIONER OF OATHS

I certify that the deponent has acknowledged that he knows and understands the contents of this Consolidated Forensic Oversight Report, that he has no objection to taking the prescribed oath, that he considers the oath to be binding on his conscience, and that he signed this report in my presence on the date and at the place set out below.

SIGNATURE OF COMMISSIONER OF OATHS:

FULL NAME:	_____
CAPACITY:	_____
BUSINESS ADDRESS:	_____
AREA OF JURISDICTION:	_____
APPOINTMENT EXPIRY DATE:	_____
DATE:	_____
PLACE:	_____

(Affix Commissioner of Oaths stamp here)

EthicHawks Forensic Governance & Compliance Consultancy
info@ethichawks.co.za | 069 635 1435 | Cape Town, South Africa | June 2026